

Online Insomnia Intervention to Reduce Alcohol Use

NCT05630118

05/07/2026

IMPORTANT NOTE:

If you accidentally select the wrong IRB type or “Protocol Process Type” while your Initial Review (IR) application is in draft form (unsubmitted), you may change your selections. Please contact the Office of Research Integrity (ORI) at 859-257-9428, IRBsubmission@uky.edu, or [request a consult](#) to resolve any questions regarding your selections *prior* to submitting your Initial Review application.

If your **submitted IR application has been returned to you for requested revisions or additional information**, to streamline the review process **do not make changes** to your selections here **unless instructed to do so by the ORI/IRB**.

Changes to this section cannot be made after initial approval has been issued (the option is not available for MR or CR).

For guidance, see:

- [Which IRB should review my research?](#)
- [Which Protocol Process Type?](#)
- ["Getting Started"](#)

Which IRB

☐ Medical ☐ NonMedical

Protocol Process Type

☐ Exemption
☐ Expedited (Must be risk level 1)
☐ Full

The revised Common Rule expanded exemption certification category 4 for certain secondary research with identifiable information or biospecimens. The regulations no longer require the information or biospecimens to be existing. For more information see the [Exemption Categories Tool](#).

EXPEDITED CERTIFICATION

0 unresolved
comment(s)

To Be Completed Only If Protocol is to Receive Expedited Review

Applicability

- A. Research activities that (1) present no more than [*minimal risk](#) to human subjects, and (2) involve only procedures listed in one or more of the following categories, may be reviewed by the IRB through the expedited review procedure authorized by 45 CFR 46.110 and 21 CFR 56.110. The activities listed should not be deemed to be of minimal risk simply because they are included on this list. Inclusion on this list merely means that the activity is eligible for review through the expedited review procedure when the specific circumstances of the proposed research involve no more than minimal risk to human subjects.
- B. The categories in this list apply regardless of the age of subjects, except as noted.
- C. The expedited review procedure may not be used where identification of the subjects and/or their responses would reasonably place them at risk of criminal or civil liability or be damaging to the subjects' financial standing, employability, insurability, reputation, or be stigmatizing, unless reasonable and appropriate protections will be implemented so that risks related to invasion of privacy and breach of confidentiality are no greater than minimal.
- D. The expedited review procedure may not be used for classified research involving human subjects.
- E. IRBs are reminded that the standard requirements for informed consent (or its waiver, alteration, or exception) apply regardless of the type of review—expedited or convened—utilized by the IRB.

**“Minimal risk” means that the probability and magnitude of harm or discomfort anticipated in the research are not greater in and of themselves from those ordinarily encountered in daily life or during the performance of routine physical or psychological examination or tests. 45 CFR 46.102(i)*

Check the appropriate categories that apply to your research project:

- ☐ Study was originally approved by the full IRB at a convened meeting.
- ☐ 1) Clinical studies of drugs and medical devices only when condition (a) or (b) is met.
- A. Research on drugs for which an investigational new drug application is not required. (Note: Research on marketed drugs that significantly increases the risks or decreases the acceptability of the risks associated with the use of the product is not eligible for expedited review.)
- B. Research on medical devices for which (i) an investigational device exemption application is not required*; or (ii) the medical device is cleared/approved for marketing and the medical device is being used in accordance with its cleared/approved labeling.**

* Study must meet one of the IDE Exempt categories listed on the Device Form Attachment.

** An approved Device used in research according to its approved labeling is considered Exempt from IDE requirements.

NOTE: Select Category 1 for compassionate use medical device applications or individual patient expanded access investigational drug applications for which FDA has waived the requirement for full review.

- ☐ 2) Collection of blood samples by finger stick, heel stick, ear stick, or venipuncture as follows:
- A. From healthy, nonpregnant adults who weigh at least 110 pounds. For these subjects, the amounts drawn may not exceed 550 ml in an 8 week period and collection may not occur more frequently than 2 times per week; or
- B. From other adults and children* considering the age, weight, and health of the subjects, the collection procedure, the amount of blood to be collected, and the frequency with which it will be collected. For these subjects, the amount drawn may not exceed the lesser of 50 ml or 3 ml per kg in an 8 week period and collection may not occur more frequently than 2 times per week.

NOTE: Intravenous (IV), Port, Central, or any other lines are NOT eligible under this category even if the research involves “minimal risk”.

*In Kentucky, “child/children” refers to all individuals less than 18 years of age unless the individual(s) is/are legally emancipated. (See [Informed Consent SOP](#) for discussion of “Emancipated Individuals” under Kentucky state law.) Individuals less than 18 years of age who are not emancipated meet the federal definition for “child” (e.g., DHHS, FDA, and U.S. Department of Education). Children are defined in the HHS regulations as “persons who have not attained the legal age for consent to treatments or procedures involved in the research, under the applicable law of the jurisdiction in which the research will be conducted.” If conducting research outside the state of Kentucky, you are responsible for complying with applicable state law.

- ☐ 3) Prospective collection of biological specimens for research purposes by noninvasive means. Examples:

- A. Hair and nail clippings in a nondisfiguring manner;
- B. Deciduous teeth at time of exfoliation or if routine patient care indicates a need for extraction;
- C. Permanent teeth if routine patient care indicates a need for extraction;
- D. Excreta and external secretions (including sweat);
- E. Uncannulated saliva collected either in an unstimulated fashion or stimulated by chewing gumbase or wax or by applying a dilute citric solution to the tongue;
- F. placenta removed at delivery;
- G. Amniotic fluid obtained at the time of rupture of the membrane prior to or during labor;
- H. Supra- and subgingival dental plaque and calculus, provided the collection procedure is not more invasive than routine prophylactic scaling of the teeth and the process is accomplished in accordance with accepted prophylactic techniques;
- I. Mucosal and skin cells collected by buccal scraping or swab, skin swab, or mouth washings;
- J. Sputum collected after saline mist nebulization.

☐ 4) Collection of data through noninvasive procedures (not involving general anesthesia or sedation) routinely employed in clinical practice, excluding procedures involving x-rays or microwaves. Where medical devices are employed, they must be cleared/approved for marketing. (Studies intended to evaluate the safety and effectiveness of the medical device are not generally eligible for expedited review, including studies of cleared medical devices for new indications.) Examples:

- A. Physical sensors that are applied either to the surface of the body or at a distance and do not involve input of significant amounts of energy into the subject or an invasion of the subject's privacy;
- B. Weighing or testing sensory acuity;
- C. Magnetic resonance imaging;
- D. electrocardiography, electroencephalography, thermography, detection of naturally occurring radioactivity, electroretinography, ultrasound, diagnostic infrared imaging, doppler blood flow, and echocardiography;
- E. moderate exercise, muscular strength testing, body composition assessment, and flexibility testing where appropriate given the age, weight, and health of the individual.

☐ 5) Research involving materials (data, documents, records, or specimens) that have been or will be collected solely for non-research purposes (such as medical treatment or diagnosis) as well as research involving existing information or specimens that were previously collected for research purposes, provided they were not collected for the currently proposed research. (Note: Some research in this category may qualify for Exempt review. This listing refers only to research that is not exempt.) (Note: If submission includes materials previously collected for either non-research or research purposes in a protocol for which IRB approval expired, you may check Category 5. However, a separate category must also be selected for prospective collection of data/specimens obtained solely for research purposes)

☒ 6) Collection of data from voice, video, digital, or image recordings made for research purposes.

☒ 7) Research on individual or group characteristics or behavior (including, but not limited to, research on perception, cognition, motivation, identity, language, communication, cultural beliefs or practices, and social behavior) or research employing survey, interview, oral history, focus group, program evaluation, human factors evaluation, or quality assurance methodologies. (Note: Some research in this category may be exempt from the HHS regulations for the protection of human subjects. This listing refers only to research that is not exempt.)

CONTINUATION REVIEW/FINAL REVIEW

1 unresolved
comment(s)

Based on your responses to the Continuation Review/Final Review questions, to be in accord with federal policy a final review report must be submitted to properly CLOSE OUT your protocol.

IF YOU WISH TO EXTEND YOUR IRB APPROVAL PERIOD, update your 'Anticipated Ending Date of Research Project' under the Project Information section and include any other supportive documentation for continuation of your study [NOTE: If you wish for your IRB approval to continue, but you do not request an extension and complete and submit your materials in a timely manner, IRB approval will expire at the end of the current approval period.].

To initiate your continuation review (CR)/annual administrative review (AAR), or properly close your study, complete this section and update/correct all other sections of your IRB application as applicable.

*****IMPORTANT***** Before leaving this page to update other sections of your application, be sure to **SAVE** this section first.

If you have any questions, please contact the Office of Research Integrity at 859-257-9428 or email IRBsubmission@uky.edu



1. Status of the Research

Check the statement(s) that best describe(s) the current status of your research:

- ☐ No subjects have enrolled to date.
- ☐ Recruitment and/or enrollment of new subjects or review of records/specimens continue.
- ☐ Study is closed to enrollment, but subjects still receive research-related interventions (e.g., treatment, blood draws).
- ☐ Study enrollment is permanently closed; subjects have completed all research-related interventions; and the study remains active only for long-term follow-up of subjects (see Tool Tip above for info on long-term follow-up of subjects).*
- ☐ Research has progressed to the point that it involves 1) Data analysis, including analysis of identifiable private information or identifiable biospecimens; and/or 2) Accessing follow-up clinical data from procedures that subjects would undergo as part of clinical care.*
 - ☒ The remaining research activities are limited only to data analysis. There is access to records or specimens either directly or through codes or links to the data.*
 - ☐ The remaining research activities are limited only to data analysis. There is no subject/record/specimen identifying codes or links to the data; the researcher or research team cannot readily ascertain the subject's identity.*
 - ☐ All study activities are complete. IRB approval can be inactivated.

*Possibility that review will move from Full to Expedited.

2. If subjects have been enrolled within the last year, and the IRB approved a consent/assent form for your study:

Please attach a complete, signed copy for the last two subjects enrolled with **each** consent/assent form/HIPAA form since the last annual review.

(Example: If 3 different approved consent forms were used since the last annual review, please provide the two most recent signed copies of each version for a total of six.)

Attachments

3. Informed Consent

If the study is **open to subject enrollment**, please go to the **Informed Consent** section of the **E-IRB Application** and verify **attachment(s) include:**

- One clean copy in PDF (without the IRB Approval stamp) of the currently approved consent/assent document(s), or,
- If requesting changes to the consent/assent document(s), submit one copy with the changes highlighted (and designate Document Type as "Highlighted"), and one clean copy in PDF (without the changes highlighted).

If the study is **open to subject enrollment** and the IRB has waived the requirement to document informed consent, please go to the **Informed Consent** section of the **E-IRB Application** and verify **attachment(s) include:**

- One clean copy in PDF of the currently approved document used for the informed consent process (e.g., cover letter, phone

script), or,

- If requesting changes to the consent/assent document(s), submit one copy with the changes highlighted (and designate Document Type as "Highlighted"), and one clean copy in PDF (without the changes highlighted).

If the study is **closed to subject enrollment**, please go to the **Informed Consent** section of the E-IRB Application and remove **Informed Consent Documents designated to get an IRB approval stamp to avoid having them appear valid for enrollment**.

4. Unanticipated Problems Involving Risk to Subjects or Others/Adverse Events Summary & Assessment

Did any **problems/adverse events** occur during the last 12 months?

☐ Yes ☒ No

In the space below, provide a written summary of both unanticipated problems* and available information regarding adverse events since the last review (e.g., initial review or annual/continuing review). The amount of detail provided in such a summary will vary depending on the type of research being conducted; in many cases, such a summary could be a brief statement that there have been no unanticipated problems and that adverse events have occurred at the expected frequency and level of severity as documented in the research protocol, the informed consent document, and investigator's brochure (if applicable). **The summary must include the PI's assessment whether the problems/adverse events warrant changes to the protocol, consent process, or risk/benefit ratio.**

Note: It is the IRB's expectation that all unanticipated problems involving risk to subjects or others or related deaths requiring prompt reporting are submitted in the appropriate time frame (See Policy [\[PDF\]](#)). Your response to this Annual/Continuing Review is considered assurance that all prompt reportable problems/adverse events have been submitted for IRB review.

*For multisite studies, the written summary should describe external events determined to be unanticipated problems involving risk to subjects or others.

PROJECT INFORMATION

0 unresolved
comment(s)

Title of Project: (Use the exact title listed in the grant/contract application, if applicable).

If your research investigates any aspect of COVID-19, please include "COVID19" at the beginning of your Project Title and Short Title

**Short Title Description**

Please use a few key words to easily identify your study - this text will be displayed in the Dashboard listing for your study.



Anticipated Ending Date of Research Project: 

Maximum number of human subjects (or records/specimens to be reviewed) 

After approval, will the study be open to enrollment of new subjects or new data/specimen collection?  ☒ Yes ☐ No

Are you requesting that the UK IRB serve as the lead IRB for a multi-site study, **OR** that the UK IRB defer review to another IRB? [Click [here](#) for "IRB Reliance" help]

☒ Yes ☐ No

If "Yes," before completing your IRB application, fill out the [Reliance Request Form](#) and submit it to irbreliance@uky.edu.

PI CONTACT INFORMATION

0 unresolved
comment(s)**Principal Investigator (PI) role for E-IRB access**

The PI is the individual holding primary responsibility on the research project with the following permissions on the E-IRB application:

1. Read;
2. write/edit;
3. receive communications; and
4. submit to the IRB (IR, CR, MR, Other Review*).

If research is being submitted to or supported by an extramural funding agency such as NIH, a private foundation or a pharmaceutical/manufacturing company, the PI listed on the grant application or the drug protocol must be listed as PI here.

Please fill in any blank fields with the appropriate contact information (gray shaded fields are not editable). Required fields left blank will be highlighted in pink after you click "Save".

To change home and work addresses, go to [myUK](#) and update using the Employee Self Service (ESS) portal. If name has changed, the individual with the name change will need to submit a [Name Change Form](#) to the Human Resources Benefits Office for entering into SAP. The new name will need to be associated with the individual's Link Blue ID in SAP before the change is reflected in E-IRB. Contact the [HR Benefits Office](#) for additional information.

The Principal Investigator's (PI) contact information is filled in automatically based on who logged in to create the application.

If you are not the Principal Investigator, do NOT add yourself as study personnel.

To change the PI contact information on an application in Researcher edit status:

- click "Change Principal Investigator";
- search for the PI's name using the search feature;
- click "Select" by the name of the Principal Investigator, then "Save Contact Information".

You will automatically be added as study personnel with editing permissions to continue editing the application.

[Change Principal Investigator:](#)

First Name:	Justin	Room# & Bldg:	Kastle Hall 2070
Last Name:	Verlinden	Speed Sort#:	0044
Middle Name:	James	Dept Code:	8E120
Department:	Psychology - 8E120	Rank:	
PI's Employee/Student ID#:	12121317	Degree:	MS
PI's Telephone #:	3099484850	PI's FAX Number:	
PI's e-mail address:	justinverlinden@uky.edu	HSP Trained:	Yes
PI is R.N.	☐ Yes ☒ No	HSP Trained Date:	2/27/2023
		RCR Trained:	Yes

Do you, the PI/researcher, have a [significant financial interest](#) related to your responsibilities at the University of Kentucky (that requires disclosure per the [UK administrative regulation 7:2](#))?

☐ Yes ☒ No

RISK LEVEL

0 unresolved
comment(s)

Indicate which of the categories listed below accurately describes this protocol

- ☐ (Risk Level 1) Not greater than minimal risk
- ☐ (Risk Level 2) Greater than minimal risk, but presenting the prospect of direct benefit to individual subjects
- ☐ (Risk Level 3) Greater than minimal risk, no prospect of direct benefit to individual subjects, but likely to yield generalizable knowledge about the subject's disorder or condition.
- ☐ (Risk Level 4) Research not otherwise approvable which presents an opportunity to understand, prevent, or alleviate a serious problem affecting the health or welfare of subjects.

*"Minimal risk" means that the probability and magnitude of harm or discomfort anticipated in the research are not greater in and of themselves from those ordinarily encountered in daily life or during the performance of routine physical or psychological examination or tests.

*****For Expedited and Exempt Applications, the research activities must be Risk Level 1 (no more than minimal risk to human subjects).*****

Refer to [UK's guidance document](#) on assessing the research risk for additional information.

SUBJECT DEMOGRAPHICS

0 unresolved comment(s)

Age level of human subjects: (i.e., 6 mths.; 2yrs., etc..) to **Study Population:**

Describe the characteristics of the subject population, including age range, gender, ethnic background and health status. Identify the criteria for inclusion and exclusion.

Provide the following information:

- A description of the subject selection criteria and rationale for selection in terms of the scientific objectives and proposed study design;
- A compelling rationale for proposed exclusion of any sex/gender or racial/ethnic group;
- Justification for the inclusion of vulnerable groups such as children, prisoners, adults with impaired consent capacity, or others who may be vulnerable to coercion or undue influence.

Please consider this [FDA Guidance on Enrollment of Participants from Underrepresented Populations in Clinical Studies](#)

Male (n=75) and female (n=75) heavy drinking adults with insomnia, ages 18-50 will be recruited by posters in local health clinics and the greater Lexington community, web and social media advertisements, and word-of-mouth referrals, without regard to race or ethnicity. All participants will be from the Lexington, Louisville, or Cincinnati metropolitan areas. The minimum age of 18 was selected to exclude minors from participation in the study and the maximum age of 50 was chosen to avoid the high rates of sleep disorders in post-menopausal women. Participants will be eligible to participate if they report at least weekly binge episodes (4/5+ drinks for women/men) and moderate to severe insomnia as indicated by ISI scores greater than 14. Exclusion criteria are any self-reported history of alcohol (severe) or other substance use disorder, obstructive sleep apnea, schizophrenia, bi-polar disorder, or other psychotic spectrum disorder, no Internet access, night shift work, lack of fluency in English, pregnancy, and nursing. Eligible participants will first be stratified by sex, and then randomized using a random number generator available via the internet (<http://www.randomizer.org/>).

Attachments

Indicate the targeted/planned enrollment of the following members of minority groups and their subpopulations. Possible demographic sources: [Kentucky State Census](#), [Kentucky Race/Ethnic Table](#), [Kentucky Population Data](#).

(Please note: The IRB will expect this information to be reported at Continuation Review time for Pre-2019 FDA-regulated Expedited review and Full review applications):

Participant Demographics				
	Cisgender Man ⓘ	Cisgender Woman ⓘ	TGNB/TGE ⓘ	Unknown/Not Reported
American Indian/Alaskan Native:	1	0	0	0
Asian:			0	0
Black/African American:	6	4	0	0
Latinx:	3	6	0	0
Native Hawaiian/Pacific Islander:	0	0	0	0
White:	26	63	0	0
American Arab/Middle Eastern/North African:	0	0	0	0
Indigenous People Around the World:	0	0	0	0
More than One Race:	4	7	0	0
Unknown or Not Reported:	1	0	0	0

If unknown, please explain why:

Indicate the categories of subjects and controls to be included in the study. You may be required to complete additional forms depending on the subject categories which apply to your research. If the study does not involve direct intervention or direct interaction with subjects, (e.g., record-review research, outcomes registries), do not check populations which the research does not specifically target. For example: a large record review of a diverse population may incidentally include a prisoner or an international citizen, but you should not check those categories if the focus of the study has nothing to do with that status.

Check All That Apply (at least one item must be selected)

ADDITIONAL INFORMATION:

- ☐ Children (individuals under age 18)
- ☐ Wards of the State (Children)
- ☐ Emancipated Minors
- ☐ Students
- ☐ College of Medicine Students
- ☐ UK Medical Center Residents or House Officers
- ☐ Impaired Consent Capacity Adults
- ☐ Pregnant Women/Neonates/Fetal Material
- ☐ Prisoners
- ☐ Non-English Speaking (translated long or short form)
- ☐ International Citizens
- ☒ Normal Volunteers
- ☐ Military Personnel and/or DoD Civilian Employees
- ☐ Patients
- ☐ Appalachian Population

Please visit the [IRB Survival Handbook](#) for more information on:

- Children/Emancipated Minors
- Students as Subjects
- Prisoners
- Impaired Consent Capacity Adults
- Economically or Educationally Disadvantaged Persons

Other Resources:

- UKMC Residents or House Officers [see [requirement of GME](#)]
- [Non-English Speaking](#) [see also the E-IRB Research Description section on this same topic]
- [International Citizens](#) [DoD SOP may apply]
- [Military Personnel and/or DoD Civilian Employees](#)

Assessment of the potential recruitment of subjects with impaired consent capacity (or likelihood):

☐ Check this box if your study does NOT involve direct intervention or direct interaction with subjects (e.g., record-review research, secondary data analysis). If there is no direct intervention/interaction you will not need to answer the impaired consent capacity questions.

Does this study focus on adult subjects with any conditions that present a high *likelihood* of impaired consent capacity or *fluctuations* in consent capacity? (see examples below)

☐ Yes ☒ No

If Yes and you are not filing for exemption certification, go to "[Form T](#)", complete the form, and attach it using the button below.

Examples of such conditions include:

- Traumatic brain injury or acquired brain injury
- Severe depressive disorders or Bipolar disorders
- Schizophrenia or other mental disorders that involve serious cognitive disturbances
- Stroke
- Developmental disabilities
- Degenerative dementias
- CNS cancers and other cancers with possible CNS involvement
- Late stage Parkinson's Disease
- Late stage persistent substance dependence
- Ischemic heart disease
- HIV/AIDS
- COPD
- Renal insufficiency
- Diabetes
- Autoimmune or inflammatory disorders
- Chronic non-malignant pain disorders
- Drug effects
- Other acute medical crises

Attachments

INFORMED CONSENT/ASSENT PROCESS/WAIVER

0 unresolved
comment(s)

For creating your informed consent attachment(s), please download the most up-to-date version listed in "All Templates" under the APPLICATION LINKS menu on the left, and edit to match your research project.

Additional Resources:

- [Informed Consent/Assent Website](#)
- [Waiver of Consent vs. Waiver of Signatures](#)
- [Sample Repository/Registry/Bank Consent Template](#)

Consent/Assent Tips:

- If you have multiple consent documents, be sure to upload each individually (not all in a combined file).
- If another site is serving as the IRB for the project, attach the form as a "Reliance Consent Form" so the document will not receive a UK IRB approval stamp; the reviewing IRB will need to stamp the consent forms.
- Changes to consent documents (e.g., informed consent form, assent form, cover letter, etc...) should be reflected in a 'tracked changes' version and uploaded separately with the Document Type "Highlighted Changes".
- It is very important that only the documents you wish to have approved by the IRB are attached; DELETE OUTDATED FILES -- previously *approved* versions will still be available in Protocol History.
- Attachments that are assigned a Document Type to which an IRB approval stamp applies will be considered the version(s) to be used for enrolling subjects once IRB approval has been issued.

Document Types that do NOT get an IRB approval stamp are:

- "Highlighted Changes",
- "Phone Script", and
- "Reliance Consent Form",
- "Sponsor's Sample Consent Form".

How to Get the Section Check Mark

1. You must:
 - a) provide a response in the text box below describing how investigators will obtain consent/assent, and
 - b) check the box for at least one of the consent items and/or check mark one of the waivers
2. If applicable attach each corresponding document(s) **as a read-only PDF**.
3. If you no longer need a consent document approved (e.g., closed to enrollment), or, the consent document submitted does not need a stamp for enrolling subjects (e.g., umbrella study, or sub-study), only select "Stamped Consent Doc(s) Not Needed".
4. After making your selection(s) be sure to scroll to the bottom of this section and SAVE your work!

**Check All That Apply**

- ☐ Informed Consent Form (and/or Parental Permission Form and/or translated short form)
- ☐ Assent Form
- ☐ Cover Letter (for survey/questionnaire research)
- ☐ Phone Script
- ☐ Informed Consent/HIPAA Combined Form
- ☐ Debriefing and/or Permission to Use Data Form
- ☐ Reliance Consent Form
- ☐ Sponsor's sample consent form for Dept. of Health and Human Services (DHHS)-approved protocol
- ☒ Stamped Consent Doc(s) Not Needed

Attachments**Informed Consent Process:**

Using active voice, in the text box below, describe how investigators will obtain consent/assent. Include:

- the circumstances under which consent will be sought and obtained
- the timing of the consent process (including any waiting period between providing information and obtaining consent)

- who will seek consent
- how you will minimize the possibility of coercion or undue influence
- the method used for documenting consent
- if applicable, who is authorized to provide permission or consent on behalf of the subject
- if applicable, specific instruments or techniques to assess and confirm potential subjects' understanding of the information

Will electronic consent form/process be utilized on-site or remotely for this study?

☒ Yes ☐ No

If yes, in addition to addressing the above bullet points, describe the e-consent method and platform, including any hyperlinks, videos, or enhancements used to convey information, if applicable. Attach a representation of the e-consent with signature fields. For guidance, see the ORI [E-Consent web page](#).

Note: all individuals authorized to obtain informed consent should be designated as such in the E-IRB "Study Personnel" section of this application.

Special considerations may include:

- Obtaining consent/assent for special populations such as children, prisoners, or people with impaired decisional capacity
- *Research Involving Emancipated Individuals*
If you plan to enroll some or all prospective subjects as emancipated, consult with UK legal counsel **prior to submitting this application to the IRB**. Include research legal counsel's recommendations in the "Additional Information" section as a separate document.
- *Research Involving Non-English Speaking Subjects*
For information on inclusion of non-English speaking subjects, or subjects from a foreign culture, see IRB Application Instructions for Recruiting Non-English Speaking Participants or Participants from a Foreign Culture.
- *Research Repositories*
If the purpose of this submission is to establish a research repository describe the informed consent process. For guidance regarding consent issues, process approaches, and sample language see the [Sample Repository/Registry/Bank Consent Template](#).

Individuals interested in participating will be directed to complete the confidential web screening questionnaire. Participants will indicate consent for the web screener by checking a box. Only participants indicating consent will continue on to the screening survey itself. Qualifying participants will provide consent at two points in the study. First, they will be emailed a link to the online REDCap survey, which contains a cover letter consent form detailing risks and benefits. Participants must indicate their consent in order to participate in the study. They also have the option to opt-out. Participants will be emailed a digital copy of the consent form following completion of the REDCap survey. Additionally, participants will be read a second consent form (included in the phone interview script) at the beginning of the 6-month follow-up phone interview. They will be reminded of their rights as research participants and potential risks. Participants must offer verbal consent prior to the interview taking place. The protocol, including consent forms, will be approved by the IRB. Subjects are informed that they do not have to participate if any procedure causes them discomfort. Participants will have the email and phone contact information of the research assistant, the UK Office of Research Integrity, and PI Moloney in the event that they wish to make confidential complaints or request information about the study.

☐ Request for Waiver of Informed Consent Process

If you are requesting IRB approval to waive the requirement for the informed consent process, or to alter some or all of the elements of informed consent, complete, Section 1 and Section 2 below.

Note: The IRB does not approve waiver or alteration of the consent process for greater than minimal risk research, except for planned emergency/acute care research as provided under FDA regulations. Contact ORI for regulations that apply to single emergency use waiver or acute care research waiver (859-257-9428).

SECTION 1.

Check the appropriate item:

☐ I am requesting a waiver of the requirement for the informed consent process.

☐ I am requesting an alteration of the informed consent process.

If you checked the box for this item, describe which elements of consent will be altered and/or omitted, and justify the alteration.

SECTION 2.

Explain how each condition applies to your research.

a) The research involves no more than minimal risk to the subject.

b) The rights and welfare of subjects will not be adversely affected.

c) The research could not practicably be carried out without the requested waiver or alteration.

d) Whenever possible, the subjects or legally authorized representatives will be provided with additional pertinent information after they have participated in the study.

e) If the research involves using or accessing identifiable private information or identifiable biospecimens, the research could not practicably be carried out without using such information or biospecimens in an identifiable format.

- Private information/specimens are “identifiable” if the investigator may ascertain the identity of the subject or if identifiers are associated with the information (e.g., medical records). This could be any of the [18 HIPAA identifiers](#) including [dates of service](#).
- If not using identifiable private information or identifiable biospecimens, insert N/A below.

If you are requesting IRB approval to waive the requirement for signatures on informed consent forms, **your research activities must fit into one of three regulatory options:**

1. The only record linking the participant and the research would be the consent document, and the principal risk would be potential harm resulting from a breach of confidentiality (e.g., a study that involves participants who use illegal drugs).
2. The research presents no more than minimal risk to the participant and involves no procedures for which written consent is normally required outside of the research context (e.g., a cover letter on a survey, or a phone script).
3. The participant (or legally authorized representative) is a member of a distinct cultural group or community in which signing forms is not the norm, the research presents no more than minimal risk to the subject, and there is an appropriate alternative mechanism for documenting that informed consent was obtained.

Select the option below that best fits your study.

*If the IRB approves a waiver of signatures, participants must still be provided oral or written information about the study. To ensure you include required elements in your consent document, use the **Cover Letter Template** as a guide. There is an [English](#) and a [Spanish](#) version.*



Option 1

Describe how your study meets these criteria:

a) The only record linking the participant and the research would be the consent document:

b) The principal risk would be potential harm resulting from a breach of confidentiality (i.e., a study that involves subjects who use illegal drugs).

Under this option, each participant (or legally authorized representative) must be asked whether (s)he wants to sign a consent document; if the participant agrees to sign a consent document, only an IRB approved version should be used.

Option 2

Describe how your study meets these criteria:

a) The research presents no more than minimal risk to the participant:

Our research presents no more than minimal risk to participants because the probability and magnitude of discomfort from this research is not greater than what is ordinarily encountered in daily life, or that which might be encountered during routine psychological tests.

b) Involves no procedures for which written consent is normally required outside of the research context (i.e. a cover letter on a survey, or a phone script):

Our research involves an online survey, and a telephone interview, procedures for which obtaining written consent is not normally required outside of research and we have created a pre-screening cover letter, a survey cover letter, a telephone interview cover letter, and phone script in accordance with UK IRB guidelines.

Option 3

Describe how your study meets these criteria:

a) The subject (or legally authorized representative) is a member of a distinct cultural group or community in which signing forms is not the norm.

b) The research presents no more than minimal risk to the subject.

c) There is an appropriate alternative mechanism for documenting that informed consent was obtained.

STUDY PERSONNEL

0 unresolved comment(s)

Do you have study personnel who will be assisting with the research?

After selecting 'Yes' or 'No' you must click the 'Save Study Personnel Information' button. ⓘ

☒ Yes ☐ No

Manage Study Personnel

Identify other study personnel assisting in research project:

- The individual listed as PI in the 'PI Contact Information' section should NOT be added to this section.
- If the research is required for a University of Kentucky academic program, the faculty advisor is also considered study personnel and should be listed below. ***Residents and students who are PI's are encouraged to designate the faculty advisor or at least one other individual as a contact with an editor role (DP).***
- Role: DP = Editor (individual can view, navigate, and edit the application for any review phase (IR, CR/FR, MR) or 'Other Review', and submit Other Reviews on behalf of the PI.)
- Role: SP = Reader (individual can view and navigate through the currently approved application only.)

To add an individual via the below feature:

- Search for personnel;
- Click "select" by the listing for the person you want to add;
- For each person, specify responsibility in the project, whether authorized to obtain informed consent, AND denote who should receive E-IRB notifications (contact status).

NOTE: Study personnel must complete human subject protection (HSP) and Responsible Conduct of Research (RCR) training before implementing any research procedures. For information about training requirements for study personnel, visit UK's [HSP FAQ page](#), the [RCR Home](#) page, or contact ORI at 859-257-9428. If you have documentation of current HSP training other than that acquired through UK CITI, you may submit it to ORI (HSPTrainingSupport@uky.edu) for credit.

Study personnel assisting in research project: ⓘ

Last Name	First Name	Responsibility In Project	Role	A C	Contact	Degree	StatusFlag	(HSP)	(HSP)Date	(RCR)	Removed?	Last Updated	SFI	Active
Belka	Thomas	Project Assistance/Support	SP	N	N		S	Y	02/15/2024	Y	N	10/16/2024	N	Y
Moloney	Mairead	Co-Investigator	DP	Y	Y	PhD	N	Y	03/03/3000	N	N	08/26/2024	N	Y
Padgett	Kelsey	Project Assistance/Support	DP	Y	N		P	Y	04/06/2023	Y	N	11/15/2023	N	Y
Trentmann	Virginia	Project Assistance/Support	SP	N	N		S	Y	02/15/2024	Y	N	10/16/2024	N	Y
Vsevolozhskaya	Olga	Faculty Advisor	DP	N	Y	PhD	P	Y	10/23/2024	Y	N	03/11/2024	N	Y
Weafer	Jessica	Co-Investigator	SP	Y	Y	PhD	N	Y	03/03/3000		N	08/15/2024	N	Y
Bansal	Pevitr	Project Assistance/Support	SP	Y	N	M.A.	S	N	04/02/2022		Y	07/14/2022	N	N
Barthel	Avrie	Data Collection	SP	N	N		S	Y	10/03/2022		Y	08/15/2024	N	N
Brooks	Alyssa	Co-Investigator	DP	Y	N		N	Y	03/03/3000		Y	07/22/2020	N	Y
Brown	Jaime	Study Coordinator	DP	Y	Y	B.A.	P	Y	11/09/2023	Y	Y	04/24/2023	N	Y
Burand	Calisse	Project Assistance/Support	DP	Y	Y		P	Y	09/03/2023		Y	09/28/2022	N	N
Count	Cassidy	Project Assistance/Support	SP	N	N		P	Y	08/10/2023	Y	Y	03/11/2024	N	Y
Elkins	Anjeli	Project Assistance/Support	SP	Y	N	M.A.	P	Y	08/13/2023		Y	03/10/2022	N	Y
Eng	Ashley	Project Assistance/Support	SP	Y	N	M.A.	P	Y	01/21/2023	N	Y	03/10/2022	N	Y
Goh	Patrick	Project Assistance/Support	SP	N	N	M.A.	P	N	03/07/2022		Y	03/11/2024	N	N
Gurney	Elise	Project Assistance/Support	DP	N	N		P	Y	03/30/2023		Y	05/14/2024	N	N
Martel	Michelle	Project Assistance/Support	SP	Y	N	Ph.D.	P	Y	02/28/2023	Y	Y	03/11/2024	N	Y
Moloney	Mairead	Co-Investigator	SP	Y	Y	PhD	N	Y	03/03/3000		Y	08/26/2024	N	Y
Naylor	Kelsey	Project Assistance/Support	SP	Y	N		S	N	08/25/2020		Y	09/28/2022	N	N
Robinson	Layne	Project Assistance/Support	SP	Y	N		S	Y	08/02/2024	Y	Y	09/28/2022	N	Y
Smith	Caitlyn	Project Assistance/Support	SP	N	N		N	Y	03/03/3000		Y	09/28/2022	N	Y
Smith	Caitlyn	Project Assistance/Support	SP	Y	N		P	N	08/04/2021		Y	09/28/2022	N	Y
Smoot	Holly	Project Assistance/Support	SP	Y	N		P	N	08/17/2021		Y	09/03/2021	N	N
Strzelecki	Ashley	Project Assistance/Support	SP	Y	N		P	Y	05/31/2023		Y	03/11/2024	N	N

Last Name	First Name	Responsibility In Project	Role	A C	Contact	Degree	StatusFlag	(HSP)	(HSP)Date	(RCR)	Removed?	Updated
Thaxton	Melina	Project Assistance/Support	SP	Y	N	B.A.	P	N	02/26/2020		Y	03/10/2022 N N
Weafer	Jessica	Co-Investigator	DP	N	Y	PhD	P	N	06/04/2021		Y	08/25/2023 N N
West	Kathleen	Project Assistance/Support	DP	Y	N		P	N	07/06/2022	Y	Y	11/15/2023 N Y
Winkel	Fiona	Project Assistance/Support	SP	Y	N		S	N	08/26/2020		Y	09/28/2022 N N

RESEARCH DESCRIPTION

0 unresolved
comment(s)

You may attach a sponsor's protocol pages in the "Additional Information" section and refer to them where necessary in the Research Description. However, each prompt that applies to your study should contain at least a summary paragraph.

Pro Tips:

- Save your work often to avoid losing data.
- Use one of the attachment buttons in this section or under the Additional Information section to include supplemental information with your application. During the document upload process, you will be able to provide a brief description of the attachment.

Background

Include a brief review of existing literature in the area of your research. You should identify gaps in knowledge that should be addressed and explain how your research will address those gaps or contribute to existing knowledge in this area. For interventional research, search PubMed and ClinicalTrials.gov for duplicative ongoing and completed trials with same condition and intervention(s).

An estimated 15 million people in the United States have alcohol use disorder (AUD). Of those, at least 40% suffer from insomnia. AUD and insomnia have a bidirectional and feed-forward relationship, such that excessive alcohol consumption negatively impacts sleep, and disrupted sleep contributes substantially to alcohol-related problems. This association may be particularly pronounced in women, as women have a 40% higher insomnia risk than men, and incidence of AUD is increasing among women at alarming rates. Initial studies show that treating insomnia in individuals with AUD can effectively disrupt this cycle and reduce AUD symptomatology. Importantly, insomnia often precedes problematic drinking in both adolescents and adults, and thus may play a causal role in the onset of AUD. As such, it is possible that insomnia treatment in high-risk heavy drinkers could also serve to prevent the transition to AUD. However, the degree to which treating insomnia reduces alcohol use in heavy drinkers is not known. Cognitive behavioral therapy for insomnia (CBT-I) is the recommended first-line insomnia treatment. CBT-I trained practitioners, however, are rare and CBT-I typically requires 6-8 office visits which may not be covered by insurance. One solution to these obstacles is the implementation of efficacious, accessible electronic cognitive behavioral therapy for insomnia (e-CBT-I). Sleep Healthy Using the Internet (SHUTi) is a well-validated version of e-CBT-I comprised of 6 weekly educational modules and daily sleep diaries. SHUTi improves sleep in adults with insomnia, adults with asthma, cancer survivors, and Appalachian women. Moreover, our research team and others have shown that SHUTi has positive effects on sleep-associated health behaviors it does not directly target, including depression, which is highly comorbid with both insomnia and AUD, maladaptive coping, perceived stress, and sleep medication use. As such, SHUTi holds promise for reducing alcohol use and is currently being piloted among treatment-seeking adults diagnosed with AUD by our NIH-based project consultant (Brooks, 18-CC-0079, NCT03493958). The current study will be the first to test the degree to which SHUTi may also decrease alcohol use in heavy drinkers, and potentially prevent the transition from heavy drinking to AUD. Our long-term goal is to implement successful, technologically-facilitated, self-administered interventions to reduce alcohol consumption and improve sleep. The objective of this application is to determine the effectiveness of SHUTi in improving sleep and reducing alcohol use among heavy drinkers with insomnia. Our central hypothesis is that SHUTi will reduce alcohol use and improve sleep and mood in this population. The rationale for this mixed methods proposal is that effective, nonpharmacologic treatments are necessary to stem the highly comorbid public health problems of AUD and insomnia. If successful, SHUTi will represent a novel and easily accessible intervention for reducing alcohol intake among high-risk heavy drinkers.

Objectives

List your research objectives. Please include a summary of intended research objectives in the box below.

Specific Aim 1: Determine the effectiveness of SHUTi to reduce alcohol consumption among heavy drinkers with insomnia. Based on evidence that insomnia treatment may reduce alcohol use in treatment-seeking individuals with AUD, we hypothesize that SHUTi will reduce alcohol use among heavy drinkers.

Specific Aim 2: Determine the effectiveness of SHUTi to decrease insomnia severity and improve sleep and mood among heavy drinkers with insomnia. Based on data collected by our team and others we hypothesize that SHUTi will decrease insomnia severity and improve sleep quality and mood among heavy drinkers.

Exploratory Aim: Explore sex differences in the effectiveness of SHUTi to reduce alcohol consumption and improve sleep and mood. This study is a collaboration between two Early Stage Investigators that combines MPI Dr. Weafer's expertise in alcohol addiction and MPI Dr. Moloney's expertise in insomnia and implementing the SHUTi intervention. Findings from the study will provide critical preliminary data showing whether SHUTi is effective at reducing alcohol consumption and improving sleep among heavy drinkers. This research is an important step towards implementing a wide-reaching, easily accessible, non-pharmacologic intervention with the potential to reduce alcohol use and improve sleep in heavy drinkers.

Study Design

Describe and explain the study design (e.g., observational, secondary analysis, single/double blind, parallel, crossover, deception, etc.).

- *Clinical Research*: Indicate whether subjects will be randomized and whether subjects will receive any placebo.
- *Community-Based Participatory Research*: If you are conducting [community-based participatory research \(CBPR\)](#), describe

strategies for involvement of community members in the design and implementation of the study, and dissemination of the study.

- **Qualitative research:** Indicate ranges where flexibility is needed, if a fixed interview transcript is not available, describe interview topics including the most sensitive potential questions.
- **Research Repositories:** If the purpose of this submission is to establish a Research Repository (bank, registry) and the material you plan to collect is already available from a commercial supplier, clinical lab, or established IRB approved research repository, provide scientific justification for establishing an additional repository collecting duplicate material. Describe the repository design and operating procedures. For relevant information to include, see the [UK Research Biospecimen Bank Guidance](#) or the [UK Research Registry Guidance](#).

Men (n=75) and women (n=75) aged 18-50 who report heavy drinking [at least weekly binge drinking (4/5+ drinks per episode for women/men) over the past 90 days] and insomnia will be recruited throughout Kentucky, Ohio, Indiana, and Florida. Half will be randomly assigned to receive the SHUTi intervention; the other half will receive access to a web-based insomnia education program. All participants will access the Internet via a personal computer, tablet, or Smartphone. All participants will complete a guided drinking calendar on zoom and an online survey comprised of validated sleep and alcohol use scales at baseline and three time points: T1 (at the end of the 9 week intervention period), T2 (3-months post-intervention), and T3 (6 months post-intervention). Participants will additionally complete daily online diaries of sleep and alcohol use at baseline, T1, and T3 and a semi-structured telephone interview at T3.

Attachments

Subject Recruitment Methods & Advertising

Describe how the study team will identify and recruit subjects. Please consider the following items and provide additional information as needed so that the IRB can follow each step of the recruitment process.

- How will the study team identify potential participants?
- Who will first contact the potential subjects, and how?
- Will you use advertisements? If so, how will you distribute those?
- How and where will the research team meet with potential participants?
- If applicable, describe proposed outreach programs for recruiting women, minorities, or disparate populations.
- How you will minimize undue influence in recruitment?
- Attach copies of all recruiting and advertising materials (emails, verbal scripts, flyers, posts, messages, etc.).

For additional information on recruiting and advertising:

- [IRB Application Instructions - Advertisements](#)
- [PI Guide to Identification and Recruitment of Human Subjects for Research](#)

The study will recruit subjects through flyers, posters, Research Spotlights, and ads placed on campus and in the surrounding community and region. (Study Team will place/remove ads), including but not limited to the UK Medical Center, UK Clinics, Good Samaritan Hospital, Student Center, UHS, the 5 UK Center for Clinical and Translational Research wall mounts, Cardinal Hill, monitor screens, and area facilities and businesses. Recruitment ads will also appear on Craig'sList. We will also recruit subjects using social media, including Facebook and Instagram. Recruitment will also take place via ResearchMatch. We will adhere to UK's guidelines for recruitment using social media. Specifically, we will adhere to the online platform's terms of use and conditions; we will obtain IRB approval before advertising over social media; we will not post ads directly on individuals' pages; we will limit postings on craigslist to one category; and we will not interact with closed groups or online forums without first obtaining permission. The study will employ a pre-screening cover letter and eligibility survey to determine if a volunteer meets basic inclusion/exclusion criteria. We will administer the eligibility survey on UK's REDCap which provides HIPAA compliant storage on UK servers and encrypted transmission of survey responses. The data collected from the prescreen survey will be used only to determine eligibility, and will not be analyzed or used for future research purposes. The link will be included in study information sent to ResearchMatch participants who have indicated interest in the study. Before redirecting the volunteer outside of ResearchMatch and to the REDCap survey, the volunteer is once again asked to confirm their interest in completing the pre-screening cover letter and survey. Prospective participants seeing a recruitment advertisement will contact the study initially by phone call, text message, or email, and will be referred to the RedCap link so they can complete a preliminary screening questionnaire. Dr. Weafer has extensive experience recruiting heavy drinking adults in and around Lexington and Dr. Moloney has successfully recruited adults with insomnia in rural Appalachian Kentucky. This study will be advertised on recruitment internet webpages in digital or video form (e.g., UKclinicalresearch.com, ResearchMatch.org, CenterWatch.com, CISCPR, UK, CCTS and may utilize Google Adwords). The study will be promoted via social media, including Facebook boost ads, UK_CCTS Facebook, UK_CCTS Twitter, UK_CCTS Instagram, UK and UKHC social media, and departmental/lab pages. If advertised on UKclinicalresearch.com, the online study flyer will include an option for interested individuals to enter and submit their contact information, they will be asked whether study team can contact them (Yes or No) via study-related text messages, and CCTS will also ask, 'How did you learn about the study?'. Internet and social media recruitment will follow the terms of use for each site utilized. The study will also be promoted through UK HC monitor screens. Potential participants may be identified from registry databases, including but not limited to ResearchMatch.org, Wellness Health and You, Sanders Brown Center on Aging, Infectious Disease, Dentistry, and the Markey Cancer Center. The CCTS attends outreach activities to promote research participation in general (e.g., Roots & Heritage Festival, Latino Festival, Eastern Kentucky University, Transylvania Health fairs, etc.) and will bring all relevant study flyers that are enrolling participants. This study may also go out on email distribution, listservs, or e-newsletters, e.g., the CCTS list serv, Markey Cancer Affiliates list servs, ResearchMatch.org, Wednesday's Word, Kentucky Office of Rural Health (KORH), Appalachian Translational Research Network (ATRN), etc. We may utilize physician referral letters to community physicians for patient recruitment; however, a modification request will be submitted to the IRB before this method of recruitment is implemented. Articles and interviews about the researchers and research study may be promoted via UKNow, Kentucky living, and

other media outlets. Research and study-related articles published on UKNow may contain standard language directed at individuals on where to read more about research and current studies. UKPR, UK HealthCare marketing or the CCIS PRS may create videos to promote research, researchers and their studies to local, regional and national media venues and on internal hospital monitors. UK HealthCare may place study recruitment flyers on their internal and external racks (e.g., UK pharmacies, clinics, UK Libraries and Lexington Libraries) or on digital monitors. Participants may be recruited using newsletters, such as In the Loop, Health Matters, Making a difference, and external news letters. The study may also be advertised through UKPR and UKHC outreach activities. All of these sources will be used to recruit participants from the Lexington area and surrounding counties. Social media ads will target participants throughout Kentucky, Ohio, Indiana, and Florida. Advertisements will invite heavy drinkers (age 18 - 50) to participate in a study on insomnia and alcohol consumption. The investigators have extensive experience using this recruitment procedure in communities for alcohol and other drug studies. All advertisements will be approved by the UK Institutional Review Board (IRB) and the UK Office of Public Relations.

In addition, we will utilize the services of a research subject recruitment firm called BuildClinical. BuildClinical is a technology company that enables academic researchers to engage the specific populations needed for their studies. They utilize machine-learning and data mining to identify specific patient groups and communities across platforms (e.g., social media, health websites, etc.) and they target advertising toward them. Advertisements link to a customized landing page that explains more about the study, and a pre-screen form that helps determine someone's eligibility. BuildClinical will then send us information on anyone who is deemed eligible based on the pre-screen.

Attachments

Attach Type	File Name
Advertising	PODS flyer_v2_PR_approved.pdf
Advertising	PODS flyer QR code PR edit STAMPED.pdf
Advertising	PODS PSYCH-071 research match PR STAMPED irb 60446.pdf
Advertising	PSYCH-071a researchmatch-STAMPED.pdf
Advertising	PSYCH-071a social media-STAMPED.pdf
Advertising	PSYCH-071a_flyer-STAMPED.pdf
Advertising	PSYCH-071a_MON-STAMPED.pdf
Advertising	PSYCH-071b researchmatch-STAMPED.pdf
Advertising	PSYCH-071b social media-STAMPED.pdf
Advertising	PSYCH-071b_flyer-STAMPED.pdf
Advertising	PSYCH-071b_MON-STAMPED.pdf
Advertising	BCFS00690-UK-Moloney-AUD [Landing Page]-STAMPED.pdf
Advertising	BCFS00690-UK-Moloney-AUD Screening Form-STAMPED.pdf
Advertising	BCFS00690-UK-Moloney-AUD-Ad2-STAMPED.pdf
Advertising	BCFS00690-UK-Moloney-AUD-Feed-STAMPED.pdf
Advertising	Indiana-BCFS00690-UK-Moloney-AUD-AdCopy (1)-STAMPED.pdf
Advertising	Kentucky-BCFS00690-UK-Moloney-AUD-AdCopy (1)-STAMPED.pdf
Advertising	Ohio-BCFS00690-UK-Moloney-AUD-AdCopy (1)-STAMPED.pdf

Research Procedures

Describe how the research will be conducted.

- What experience will study participants have?
- What will study participants be expected to do?
- How long will the study last?
- Outline the schedule and timing of study procedures.
- Provide visit-by-visit listing of all procedures that will take place.
- Identify all procedures that will be carried out with each group of participants.
- Describe deception and debrief procedures if deception is involved.

Screening: Participants will complete an online screening survey in REDCap. They are told at the beginning of this survey that they will be contacted by phone if they are eligible to participate in the study.

Baseline Assessment: Participants will complete an online survey in REDCap containing validated measures of alcohol-related problems (AUDIT), alcohol valuation (APT) and sleep (ISI, Pittsburgh Sleep Quality Index). We will also collect data on common comorbidities including stress (Perceived Stress Scale) and depression (Center for Epidemiologic Studies Depression Scale). Moloney used these assessments in her previous SHUTi pilot. Additionally, we will gather information on Circadian rhythms (CMQ) and sleep medication use. Following completion of these surveys, participants will fill out a Timeline Follow-Back (TLFB), a drinking calendar which allows participants to report their drinking habits for each of the previous 30 days. This TLFB will be completed over zoom and guided by a member of the research team to ensure adequate detail and accuracy. Finally, participants will then complete an 11-item daily sleep and alcohol use diary for ten days.

SHUTi (Treatment): To offer scheduling flexibility, participants will have 9 weeks to complete 6 once-weekly educational “cores.” Cores consist of: insomnia overview, sleep restriction, stimulus control, cognitive restructuring, sleep hygiene, and relapse prevention, and last approximately 45 minutes each. Participants will receive weekly objectives, as well as review and feedback on the previous core’s assignments and sleep diary data, new material, assignments, and a summary. These features are supplemented with interactive features (i.e., personalized goal-setting, video vignettes). Participants receive daily emails prompting them to complete the 11-item sleep and alcohol use diary daily during the intervention.

Web-based insomnia education (Control). The online education program has been used in previous SHUTi trials. Participants will be given access to a web-based program on sleep education to read at their own pace during the 9-week intervention period. The program content overlaps with SHUTi on the following topics: insomnia symptoms, impacts, and causes, basic sleep improvement strategies, and when to see a doctor. The information is akin to that found on WebMD or National Sleep Foundation website. Unlike the SHUTi program, the information is fixed, accessible immediately (no week-to-week information “unlocking”) and there are no customized bedtime or wake-time suggestions based on sleep diary data.

Participants who are randomly assigned to the intervention condition will be granted access to the program free of charge for up to six months from the start of the study. Participants who are randomly assigned to the education-only condition will be given access to the full SHUTi program three months after completing the study upon request. Similarly, participants who are randomly assigned to the intervention condition will be granted access to the education materials provided to the education-only condition participants three months after completing the study upon request.

Follow-up assessments: Participants will repeat the baseline REDCap survey containing alcohol use and sleep scales and the guided TLFB immediately following completion of the intervention and at 3-, and 6-month post intervention follow-ups. The post-intervention version of the REDCap surveys will additionally contain adherence, acceptability, and satisfaction scales. Participants will also complete an additional 10 days of daily sleep and alcohol use diaries at the post-intervention and 6-month follow-ups. At the 6-month follow-up, Moloney will lead a brief (30 min) qualitative, semi-structured interview over the telephone to reveal nuanced insights about participants’ individual experiences and the intersections of alcohol use and sleep. A second interviewer will also participate on interviews to aid in note-taking and reduce bias.

Attachments

Attach Type	File Name
ResearchProcedures	Daily_diary_prompt.docx

Data Collection & Research Materials

In this section, please provide the following:

- Describe all sources or methods for obtaining research materials about or from living individuals (such as specimens, records, surveys, interviews, participant observation, etc.), and explain why this information is needed to conduct the study.
- For each source or method described, please list or attach all data to be collected (such as genetic information, interview scripts, survey tools, data collection forms for existing data, etc.).
- If you will conduct a record or chart review, list the beginning and end dates of the records you will view.

Data to be collected will consist of demographics, alcohol use, sleep duration and quality, perceived stress, depressive symptoms, medication use, and general medical history. We will also collect data on perceived helpfulness, satisfaction, and acceptability of the

SHUTi and PE programs. All measures attached here will be completed by both the SHUTi and web-based education groups. The initial participant screen questionnaire, AUDIT, and ISI will be completed at screening. The ISI, PSQI, AUDIT, TLFB, APT, PSS, CESD-10, and sleep med questionnaire will be completed at baseline, post-intervention, 3-month follow-up, and 6-month follow-up. The CMQ will only be completed at baseline as circadian preferences are known to be stagnant. The Treatment Satisfaction Scales will be completed post-intervention.

Attachments

Attach Type	File Name
DataCollection	SIESTA Post-intervention Interview Protocol 3-11-23.docx
DataCollection	TLFB Zoom Instructions 2.16.23.docx
DataCollection	STOP-Bang.docx
DataCollection	7.Treatment_Satisfaction.docx
DataCollection	8.Composite Morningness Questionnaire.pdf
DataCollection	9. APT-Trait V1 20 NOV 2019.docx
DataCollection	Sleep_Med_Questionnaire.pdf
DataCollection	Insomnia Severity Index -ISI.pdf
DataCollection	PSQI_sleep_questionnaire_1_pg.pdf
DataCollection	AUDIT.docx
DataCollection	TLFB.docx
DataCollection	PerceivedStressScale.pdf
DataCollection	CESD-10.pdf
DataCollection	SIESTA prescreen final.pdf

Resources

Describe the availability of the resources and adequacy of the facilities that you will use to perform the research. Such resources may include:

- Staffing and personnel, in terms of availability, number, expertise, and experience;
- Computer or other technological resources, mobile or otherwise, required or created during the conduct of the research;
- Psychological, social, or medical services, including equipment needed to protect subjects, medical monitoring, ancillary care, or counseling or social support services that may be required because of research participation;
- Resources for communication with subjects, such as language translation/interpretation services.

Technological resources useful for this project include a laptop computer, statistical and qualitative analysis software (Stata 15, NVivo12), and REDCap (provided by UK CCTS). For telephone interviews, we will use Simple Recorder (an app that *is not cloud-connected*) to record interviews. As described elsewhere, participants will be informed of the recording procedures, risks, safety precautions taken, and will consent prior to any substantive questions being asked. General technological resources in the departments of Psychology and Sociology include private offices, office and laptop computers, printer/scanner, webcams, and telecommunications software (Zoom). We also plan to work closely with the UK CCTS. Their project navigation, recruitment, and REDCap services will be crucial to the success of our project.

Potential Risks & Benefits

Risks

- Describe any potential risks – including physical, psychological, social, legal, ability to re-identify subjects, or other risks. Assess the seriousness and likelihood of each risk.
- Which risks may affect a subject's willingness to participate in the study?
- *Qualitative research* - describe ethical issues that could arise while conducting research in the field and strategies you may use to handle those situations.
- Describe any steps to mitigate these risks.

Benefits

- Describe potential direct benefits to study participants – including diagnostic or therapeutic, physical, psychological or emotional, learning benefits. This cannot include incentives or payments.
- State if there are no direct benefits.
- Describe potential benefits to society and/or general knowledge to be gained.

Describe why potential benefits are reasonable in relation to potential risks. If applicable, justify why risks to vulnerable subjects are reasonable to potential benefits.

The risks to subjects are minimal and involve risks of: 1) confidentiality and 2) psychological discomfort.

1) Given the nature of online surveys, as with anything involving the Internet, we can never guarantee the confidentiality of the data while still en route to us. Participant data will be de-identified, stored on a secure server, and accessible only to IRB-approved study personnel.

2) Participants may feel uncomfortable answering some questions about sleep and alcohol use. Participants will be told that they may

skip any question that makes them uncomfortable.

The minimal risks to participants are justified by the knowledge to be gained regarding the potential for an Internet-based insomnia intervention to improve sleep and reduce alcohol use among heavy drinkers with insomnia. While there may be some benefits to study participants in terms of improved sleep and/or reduced alcohol use, establishing the feasibility, acceptability, and effectiveness of SHUTi to directly address these behaviors may be of great benefit to others by setting the groundwork for an accessible, behavioral intervention that improves multiple health behaviors in both the short and long term. Participants may benefit from the information obtained during the screening procedure, including an awareness of problematic behaviors in which they are engaging. Information about counseling/treatment options will be provided as needed.

Available Alternative Opportunities/Treatments

Describe alternative treatments or opportunities that might be available to those who choose not to participate in the study, and which offer the subject equal or greater advantages. If applicable, this should include a discussion of the current standard of care treatment(s).

There are no alternative treatments.

[Back to Top](#)

Records, Privacy, and Confidentiality

Specify where the data and/or specimens will be stored and how the researcher will ensure the privacy and confidentiality of both. Specify who will have access to the data/specimens and why they need access.

Describe how data will be managed after the study is complete:

- If data/specimens will be maintained, specify whether identifiers will be removed from the maintained information/material.
- If identifiers will not be removed, provide justification for retaining them and describe how you will protect confidentiality.
- If the data/specimens will be destroyed, verify that this will not violate [retention policies](#) and will adhere to applicable facility requirements.

If this study will use de-identified data from another source, describe what measures will be taken to ensure that subject identifiers are not given to the investigator.

If applicable, describe procedures for sharing data/specimens with collaborators not affiliated with UK.

For additional considerations:

[Return of Research Results or Incidental Research Findings](#)

[HIPAA policies](#)

[FERPA policies](#)

[Procedures for Transfer agreements](#)

[Information regarding multi-site studies](#)

[NIH Genomic Data Sharing \(GDS\) Policy](#)

[Digital Data](#)

Data will be collected exclusively through participant self-report via opt-in online, REDCap surveys and opt-in telephone interviews. Participant data will be de-identified, stored on a secure server, and accessible only to IRB-approved study personnel. Data will be kept in a password-protected file on the UK server and will be accessible only by project personnel. All information will be in the form of numerical data and used for research purposes only. Results will be reported only for groups, and no single individual will be identified. All research materials obtained from participants will be collected directly from them at the time of screening and participation. Personal identifiers, such as names, will be removed from subjects' data records. All data will be stored in coded files and locked in the laboratory area.

All research study personnel will have up-to-date CITI training. To maintain confidentiality: (1) each email address and telephone number will be copied into a new spreadsheet with no participant data, where we will assign each address a unique numerical ID code; (2) email addresses and telephone numbers will then be completely deleted from the data file and replaced only with the ID code; (3) these new data files and the email spreadsheet will be stored separately on encrypted servers, so that if the servers are breached, these files will not be linked. Emails and phone numbers will be retained for the duration of the study. Once the study is complete we will work with our UK Network administrator to assist us in securely deleting any personally identifying information. Research records, survey data, and transcripts will be stored on a password-protected, closed drive that only the research team has access to. Telephone interviews will be professionally transcribed by Rev.com, and transcribers will be instructed not to include participants' names, telephone numbers, or email addresses in the transcript. One of the investigators or the research assistant will ensure that each transcript is fully blinded before saving it onto our password protected drive. Audio recordings will be deleted once they have been transcribed. All IRB-related materials and records will be retained for a minimum of 6 years after the end of the IRB-approval period.

To protect against, or minimize any possible risks, we will follow these procedures:

- 1) Participants will be carefully screened to exclude those who are physically or psychiatrically at risk.
- 2) Participant files containing confidential information will be maintained in a locked cabinet in the PI's office. Only personnel directly connected with the study have access to this information, and these individuals are instructed in the importance and procedures for maintaining confidentiality.
- 3) Data collected in the study are identified by subject codes only, and no data will be published in a form by which the subject can be identified.

[UK IRB policies](#) state that IRB-related research records must be retained for a minimum of 6 years after study closure. Check this item to confirm that you will retain all IRB-related records for a minimum of 6 years after study closure.

Payment

Describe the incentives (monetary or other) being offered to subjects for their participation. If monetary compensation is offered, indicate the amount and describe the terms and schedule of payment. Please review [this guidance](#) for more information on payments to subjects, including restrictions and expectations.

Participants will be paid \$200 over the course of the study. To maintain motivation, post-intervention payments will be higher than initial payments. Payment points and amounts are: P1 - After completing the intake REDCap Survey, TLFB drinking calendar, and 10-day diary (\$20); P2 - After completing the post-intervention REDCap Survey, TLFB drinking calendar, and 10-day diary (at the end of the 9-week intervention) (\$60); P3 - REDCap survey and TLFB drinking calendar at 3-months post-intervention (\$30) and P4 - REDCap survey, TLFB drinking calendar, 10-day sleep diary, and semi-structured phone interview at 6-months post-intervention (\$90). Payment will be in the form of electronic gift cards, which will be emailed to participants. Individual payments will not be prorated.

Costs to Subjects

Describe any research costs which participants may be responsible for if they participate in the study (e.g., urine, HIV test).

There are no anticipated costs to subjects.

Data and Safety Monitoring

The IRB requires review and approval of data and safety monitoring plans for greater than minimal risk research or NIH-funded/FDA-regulated clinical investigations.

- If you are conducting greater than minimal risk research, or your clinical investigation is NIH-funded, describe your Data and Safety Monitoring Plan (DSMP). [Click here for additional guidance on developing a Data and Safety Monitoring Plan.](#)
- If this is a non-sponsored investigator-initiated protocol considered greater than minimal risk research, and if you are planning on using a Data and Safety Monitoring Board (DSMB) as part of your DSMP, [click here for additional guidance](#) for information to include with your IRB application.



Although this study provides only minimal risk, the PI will oversee the entire research project in order to ensure that all research activities are conducted ethically, and that the data collected are valid. All electronic data will be stored on a secured computer and transferred through secure connections; all quantitative data (survey questions, measures) will be collected through the secure web application RedCap available through UK's Center for Clinical and Translational Science. The PI's will engage in regular meetings with project staff—including Co-I's and research assistants—to monitor project activities, maintain procedures, and uphold human subjects protections. Any serious adverse events will be reported to the UK IRB.

[Back to Top](#)

Future Use and Sharing of Material (e.g., Data/Specimens/Information)

If the material collected for this study will be used by members of the research team or shared with other researchers for future studies, please address the following:

- list the biological specimens and/or information that will be kept
- briefly describe the types, categories and/or purposes of the future research
- describe any risks of the additional use
- describe privacy/confidentiality protections that will be put into place
- describe the period of time specimens/information may be used
- describe procedures for sharing specimens/information with secondary researchers
- describe the process for, and limitations to, withdrawal of specimens/data

The following deidentified data collected from this study will be submitted to the National Institute of Alcohol Abuse and Alcoholism Database (NIAAADA) at the National Institutes of Health (NIH): ISI, PSQI, AUDIT, PSS, CESDR, APT, TLFB, Sleep Med Questionnaire, and CSM. When data are uploaded to NIAAADA, each participant will have to have an associated Global Unique ID, or GUID. This identifier is created using a free tool maintained by NIAAADA by inputting information from a participant's birth certificate, specifically their name, birthdate, and city of birth.

NIAAADA is a large database where deidentified study data from many NIAAA studies are stored and managed. Researchers from around the world can request access to data from this database and, after review from NIH experts, may be granted access to the deidentified data from this study. The study data provided to NIAAADA may help researchers around the world learn more about alcohol problems and how to help others who have problems with alcohol. NIAAA will also report to Congress and on its website about the different studies using NIAAADA data. Some risks may apply, such as access to the data from unauthorized personnel, although the NIH will make every attempt to prevent this from occurring.

Participants must consent to have their data shared with NIAAADA and will not be penalized if they choose not to consent to have their

data to be shared, completing all other study materials as normal. Participants may choose to no longer have their NIAAADA at any point during the study. Participants data will not be uploaded to NIAAADA until they have fully completed the study.

Are you recruiting or expect to enroll **Non-English Speaking Subjects or Subjects from a Foreign Culture?** (does not include short form use for incidentally encountered non-English subjects)

☐ Yes ☒ No

Non-English Speaking Subjects or Subjects from a Foreign Culture

Recruitment and Consent:

Describe how information about the study will be communicated to potential subjects appropriate for their culture, and if necessary, how new information about the research may be relayed to subjects during the study.

When recruiting Non-English-speaking subjects, provide a consent document in the subject's primary language. After saving this section, attach both the English and translated consent documents in the "Informed Consent" section.

Cultural and Language Consultants:

The PI is required to identify someone who is willing to serve as the cultural consultant to the IRB.

- This person should be familiar with the culture of the subject population and/or be able to verify that translated documents are the equivalent of the English version of documents submitted.
- The consultant should not be involved with the study or have any interest in its IRB approval.
- Please include the name, address, telephone number, and email of the person who agrees to be the cultural consultant for your study.
- ORI staff will facilitate the review process with your consultant. Please do not ask them to review your protocol separately.

For more details, see the IRB Application Instructions on [Research Involving Non-English Speaking Subjects or Subjects from a Foreign Culture](#).

Local Requirements:

If you will conduct research at an international location, identify and describe:

- relevant local regulations
- data privacy regulations
- applicable laws
- ethics review requirements for human subject protection

Please provide links or sources where possible. If the project has been or will be reviewed by a local ethics review board, attach a copy in the "Additional Information/Materials" section. You may also consult the current edition of the [International Compilation of Human Research Standards](#)

Does your study involve **HIV/AIDS research and/or screening for other reportable diseases (e.g., Hepatitis**

☐ Yes ☒ No

HIV/AIDS Research

If you have questions about what constitutes a reportable disease and/or condition in the state of Kentucky, see ORI's summary sheet: "Reporting Requirements for Diseases and Conditions in Kentucky" [\[PDF\]](#).

HIV/AIDS Research: There are additional IRB requirements for designing and implementing the research and for obtaining informed consent. Describe additional safeguards to minimize risk to subjects in the space provided below.

For additional information, visit the online [IRB Survival Handbook](#) to download a copy of the "Medical IRB's requirements for Protection of Human Subjects in Research Involving HIV Testing" [D65.0000] [\[PDF\]](#), and visit the [Office for Human Research Protections web site](#) for statements on AIDS research, or contact the Office of Research Integrity at 859-257-9428.

PI-Sponsored FDA-Regulated Research

Is this an investigator-initiated study that:

- 1) involves testing a Nonsignificant Risk (NSR) Device, or
- 2) is being conducted under an investigator-held Investigational New Drug (IND) or Investigational Device Exemption (IDE)?

☐ Yes ☒ No

PI-Sponsored FDA-Regulated Research

If the answer above is yes, then the investigator assumes the regulatory responsibilities of both the investigator and sponsor. The Office of Research Integrity provides a summary list of sponsor IND regulatory requirements for drug trials [\[PDF\]](#), IDE regulatory requirements for SR device trials [\[PDF\]](#), and abbreviated regulatory requirements for NSR device trials [\[PDF\]](#). For detailed descriptions see [FDA Responsibilities for Device Study Sponsors](#) or [FDA Responsibilities for IND Drug Study Sponsor-Investigators](#).

- Describe the experience/knowledge/training (if any) of the investigator serving as a sponsor (e.g., previously held an IND/IDE); and
- Indicate if any sponsor obligations have been transferred to a commercial sponsor, contract research organization (CRO), contract monitor, or other entity (provide details or attach FDA 1571).

IRB policy requires mandatory training for investigators who are also FDA-regulated sponsors (see [Sponsor-Investigator FAQs](#)). A sponsor-investigator must complete the applicable Office of Research Integrity web based training, (drug or device) before final IRB approval is granted.

Has the sponsor-investigator completed the mandatory PI-sponsor training prior to this submission?

☒ Yes ☐ No

If the sponsor-investigator has completed equivalent sponsor-investigator training, submit documentation of the content for the IRB's consideration.

[Attachments](#)

HIPAA

0 unresolved
comment(s)Is HIPAA applicable? ☐ Yes ☒ No

(Visit ORI's [Health Insurance Portability and Accountability Act \(HIPAA\) web page](#) to determine if your research falls under the HIPAA Privacy Regulation.)

I have attached a HIPAA Waiver of Authorization. ☐ Yes ☒ No

Attachments

STUDY DRUG INFORMATION

0 unresolved
comment(s)

Drugs are articles intended for use in the diagnosis, cure, mitigation, treatment, or prevention of disease and articles (other than food) intended to affect the structure or any function of the body of man or other animals.

The term drug may include:

- FDA approved drugs,
- unapproved use of approved drugs,
- investigational drugs or biologics,
- other compounds or products intended to affect structure or function of the body, and/or
- [complementary and alternative medicine products](#) such as dietary supplements, substances generally recognized as safe (GRAS) when used to diagnose, cure mitigate, treat or prevent disease, or clinical studies of [e-cigarettes](#) examining a potential therapeutic purpose.

Does this protocol involve a drug including an FDA approved drug; unapproved use of an FDA approved drug; and/or an investigational drug?☐ Yes ☒ No

If yes, complete the questions below. Additional [study drug guidance](#).

LIST EACH DRUG INVOLVED IN STUDY IN THE SPACE BELOW

Drug Name:

Note: Inpatient studies are required by Hospital Policy to utilize [Investigational Drug Service \(IDS\) pharmacies \(Oncology or Non-Oncology\)](#). Use of IDS is highly recommended, but optional for outpatient studies. Outpatient studies not using IDS services are subject to periodic inspection by the IDS for compliance with drug accountability good clinical practices.

Indicate where study drug(s) will be housed and managed:

☐ Investigational Drug Service (IDS) UK Hospital

Other Location:

Is the study being conducted under a valid Investigational New Drug (IND) application?

☒ Yes ☐ No

If Yes, list IND #(s) and complete the following:

IND Submitted/Held by:

Sponsor: ☐Held By: Investigator: ☐Held By: Other: ☐Held By:

☐ Checkmark if the study is being conducted under FDA's Expanded Access Program (e.g., Treatment IND) or if this is an Individual Patient Expanded Access IND ([FDA Form 3926](#)).

[FDA's Expanded Access Program Information for Individual Patient Expanded Access INDs](#), and attach the following:

- [FDA Form 3926](#);
- FDA expanded access approval or correspondence;
- Confirmation of agreement from manufacturer or entity authorized to provide access to the product.

For guidance and reporting requirements at the conclusion of treatment see the [Expanded Access SOP](#).

Complete and attach the required [Study Drug Form](#) picking "Study Drug Form" for the document type. Any applicable drug documentation (e.g., Investigator Brochure; approved labeling; publication; FDA correspondence, etc.) should be attached using "Other Drug Documentation" for the document type.



Attachments

STUDY DEVICE INFORMATION

0 unresolved
comment(s)

Medical devices are intended for use in the diagnosis of disease or other conditions, or in the cure, mitigation, treatment, or prevention of disease, in man or other animals, or intended to affect the structure or any function of the body of man or other animals.

A DEVICE may be a:

- component, part, accessory;
- assay, reagent, or in-vitro diagnostic device;
- software, digital health, or mobile medical app;
- other instrument if intended to affect the structure or function of the body, diagnose, cure, mitigate, treat or prevent disease; or
- a homemade device developed by an investigator or other non-commercial entity and not approved for marketing by FDA.

For additional information, helpful resources, and definitions, see ORI's [Use of Any Device Being Tested in Research web page](#).

Does this protocol involve testing (collecting safety or efficacy data) of a medical device including an FDA approved device, unapproved use of an approved device, humanitarian use device, and/or an investigational device?

☐ Yes ☒ No

[Note: If a marketed device(s) is only being used to elicit or measure a physiologic response or clinical outcome, AND, NO data will be collected on or about the device itself, you may answer "no" above, save and exit this section, (Examples: a chemo drug study uses an MRI to measure tumor growth but does NOT assess how effective the MRI is at making the measurement; an exercise study uses a heart monitor to measure athletic performance but no safety or efficacy information will be collected about the device itself, nor will the data collected be used for comparative purposes against any other similar device).]

If you answered yes above, please complete the following questions.

LIST EACH DEVICE BEING TESTED IN STUDY IN THE SPACE BELOW

Device Name:

Is the study being conducted under a valid Investigational Device Exemption (IDE), Humanitarian Device Exemption (HDE) or Compassionate Use?

☐ Yes ☒ No

If Yes, complete the following:
IDE or HDE #(s)

IDE/HDE Submitted/Held by:

Sponsor: ☐

Held By:

Investigator: ☐

Held By:

Other: ☐

Held By:

☐ Check if this is a Treatment IDE or Compassionate Use under the Food and Drug Administration (FDA) Expanded Access program.

For Individual or Small Group Expanded Access, see [FDA's Early Expanded Access Program Information](#), and attach the following:

- FDA expanded access approval or sponsor's authorization;
- An independent assessment from an uninvolved physician, if available;
- Confirmation of agreement from manufacturer or entity authorized to provide access to the product.

For guidance and reporting requirements at the conclusion of treatment see the [Medical Device SOP](#).

Does the intended use of any research device being tested (not clinically observed) in this study meet the regulatory definition [\[FDA's PDF\]](#) of Significant Risk (SR) device?

- ☐ Yes. Device(s) being tested in this study presents a potential for serious risk to the health, safety, or welfare of a subject and (1) is intended as an implant; or (2) is used in supporting or sustaining human life; or (3) is of substantial importance in diagnosing, curing, mitigating or treating disease, or otherwise prevents impairment of human health; or (4) otherwise presents a potential for serious risk to the health, safety, or welfare of a subject.
- ☐ No. All devices being tested in this study do not present a potential for serious risk to the health, safety, or welfare of subjects/participants.

Complete and attach the required [Study Device Form](#), picking the "Study Device Form" for the document type. Any applicable device documentation (e.g., Manufacturer information; patient information packet; approved labeling; FDA correspondence, etc.) should be attached using "Other Device Documentation" for the document type.



Attachments

RESEARCH SITES

0 unresolved
comment(s)

To complete this section, ensure the responses are accurate then click "SAVE".

A) Check all the applicable sites listed below at which the research will be conducted. If none apply, you do not need to check any boxes.

UK Sites

- ☐ UK Classroom(s)/Lab(s)
- ☐ UK Clinics in Lexington
- ☐ UK Clinics outside of Lexington
- ☐ UK Healthcare Good Samaritan Hospital
- ☐ UK Hospital

Schools/Education Institutions

- ☐ Fayette Co. School Systems *
- ☐ Other State/Regional School Systems
- ☐ Institutions of Higher Education (other than UK)

***Fayette Co. School systems, as well as other non-UK sites, have additional requirements that must be addressed. See ORI's [IRB Application Instructions - Off-site Research](#) web page for details.**

Other Medical Facilities

- ☐ Bluegrass Regional Mental Health Retardation Board
- ☐ Cardinal Hill Hospital
- ☐ Eastern State Hospital
- ☐ Norton Healthcare
- ☐ Nursing Homes
- ☐ Shriner's Children's Hospital
- ☐ Veterans Affairs Medical Center
- ☐ Other Hospitals and Med. Centers

- ☐ Correctional Facilities
- ☐ Home Health Agencies
- ☐ International Sites

Research activities conducted at performance sites that are not owned or operated by the University of Kentucky (UK) or at sites that do not fall under the UK IRB's authority, are subject to special procedures for coordination of research review. Additional information is required (see [IRB Application Instructions - Off-Site Research](#) web page), including:

- A letter of support and local context is required from non-UK sites. See *Letters of Support and Local Context* on the [IRB Application Instructions - Off-Site Research](#) web page for more information.
- Supportive documentation, including letters of support, can be attached below. When attaching reliance documents, please ensure that you select the correct 'Document Type' from the drop-down menu. See below for the "Document Types" in bold, followed by examples of reliance documents for each type:
 - **Individual Investigator Agreement (IIA)**
 - A completed Individual Investigator Agreement

- IRB Approval (Non-UK)

- A Letter of Approval from a Non-UK IRB

- IRB Authorization Agreement (IAA)

- A SMART IRB Agreement
- An OHRP Agreement
- A DoD Agreement
- An IREx Reliance Notification
- Any Reliance Agreement

- Letter of Support & Local Context

- A Letter of Support from an organization at which some research activities are occurring
- Communications Plan
- Local Context Form

Please reach out to IRBReliance@uky.edu if you have any questions or concerns.

- NOTE: If the non-UK sites or non-UK personnel are engaged in the research, there are additional federal and university requirements which need to be completed for their participation. For instance, the other site(s) may need to complete their own IRB review, or a cooperative review arrangement may need to be established with non-UK sites.
- Questions about the participation of non-UK sites/personnel should be discussed with the ORI staff at (859) 257-9428.

List all other non-UK owned/operated locations where the research will be conducted:

Describe the role of any non-UK site(s) or non-UK personnel who will be participating in your research.

Please describe the plan for the management of reporting unanticipated problems, noncompliance, and submission of protocol modifications and interim results from the non-UK sites:

The OSU site will be involved in data analysis, data interpretation, and manuscript writing. Data collection will be entirely online, and no participant interactions will occur at OSU. Results will be collected on REDCap, an online platform that can be accessed by researchers from both sites.

The University of Miami site will conduct qualitative interviews with research subjects, analyze the resulting data, and assist with manuscript writing. Data collection will be completed over the phone.

Researchers at the three sites will schedule regular check-in meetings in order to discuss unanticipated problems, noncompliance, and protocol modifications.

Attachments

B) If your research involves collaboration with any sites and/or personnel outside the University of Kentucky, then it is considered multisite research and IRB reliance issues will need to be addressed. This may include national multi-center trials as well local studies involving sites/personnel external to UK. If you would like to request that the University of Kentucky IRB (UK IRB) serve as the lead IRB for your study, or if you would like the UK IRB to defer review to another IRB, please contact the IRBReliance@uky.edu.

RESEARCH ATTRIBUTES

0 unresolved
comment(s)

Instructions: For various reasons, it is necessary to determine whether your research activities meet the definition of clinical research and/or a clinical trial. Your responses to the next series of questions will make that determination. For more details on the definitions, go to ORI's [clinical research vs. clinical trial web page](#) or visit [NIH's decision tree](#) for the NIH Clinical Trial definition.

Contact the Clinical Research Support Office (CRSO) if your study provides clinical services (e.g., labs, biopsies, tissue samples, physical exams, PT, counseling) regardless of payer (grant, federal, UK, industry)), utilizes UKHC space, or meets the NIH definition of a clinical trial (thereby requiring registry with CT.gov) as your study will need to be entered in OnCore to ensure appropriate regulatory tracking and billing. Visit [CRSO FAQs](#) for more information; requests for CCTS/CRSO services can be submitted via their [service request form](#). For other questions, you can contact the CRSO Director, Jessica Heskell, at jhesk2@uky.edu.

My research activities include one or more of the following:

Patient-oriented research regarding mechanisms of human disease, therapeutic interventions, clinical studies, or development of new technologies

☐ Yes ☐ No

Material of human origin (such as tissues, specimens, and cognitive phenomena) for which an investigator (or colleague) directly interacts with human subjects

☐ Yes ☐ No

Epidemiologic or Behavioral Studies

☐ Yes ☐ No

Outcomes Research or Health Services Research

☐ Yes ☐ No

Does your research study involve one or more human subjects prospectively assigned into one or more interventions (which may include placebo or other control) to evaluate the effects of those interventions on health-related biomedical or behavioral outcomes?

☐ Yes ☐ No

Indicate the items below that apply to your research. Depending on the items applicable to your research, you may be required to complete additional forms or meet additional requirements. Contact the ORI (859-257-9428) if you have questions about additional requirements.

☐ Not applicable

Check All That Apply

- ☐ Academic Degree/Required Research
- ☒ Alcohol/Drug/Substance Abuse Research
- ☐ Biological Specimen Bank Creation (for sharing)
- ☐ Cancer Research
- ☐ CCTS-Center for Clinical & Translational Science
- ☐ Certificate of Confidentiality
- ☐ Collection of Biological Specimens for banking and use
- ☐ Community-Based Participatory Research
- ☐ Deception
- ☐ Educational/Student Records (e.g., GPA, test scores)
- ☐ Emergency Use (Single Patient)
- ☐ Gene Transfer

For additional requirements and information:

- [Cancer Research \(MCC PRMC\)](#)
- [Certificate of Confidentiality](#) (look up "Confidentiality/Privacy...")
- [CCTS \(Center for Clinical and Translational Science\)](#)
- [Clinical Research](#) (look up "What is the definition of....")
- [Clinical Trial](#)
- [Collection of Biological Specimens for Banking](#) (look up "Banks, Repositories, Registries...")
- [Collection of Biological Specimens](#) (look up "Repositories, Registries, Specimen/Tissue Banks...")
- [Community-Based Participatory Research](#) (look up "Community-Engaged...")
- [Data & Safety Monitoring Board](#) (DSMB)

*For Medical IRB: [Service Request Form](#) for CCTS DSMB

- [Data & Safety Monitoring Plan](#)
- [Deception*](#)

- ☐ Genetic Research
- ☐ NIH Genomic Data Sharing (GDS) (databases such as GWAS, dbGaP, GenBank)
- ☐ Treatment with Human Cells, Tissues, and Cellular and Tissue Based Products
- ☐ Individual Expanded Access or Compassionate Use
- ☐ International Research
- ☐ Planned Emergency Research Involving Exception from Informed Consent
- ☐ Recombinant DNA
- ☐ Registry or data repository creation
- ☐ Stem Cell Research
- ☐ Suicide Ideation or Behavior Research
- ☒ Survey Research
- ☐ Transplants
- ☐ Use, storage and disposal of radioactive material and radiation producing devices
- ☐ Vaccine Trials

*For deception research, also go to the E-IRB Application Informed Consent section, checkmark and complete "Request for Waiver of Informed Consent Process"

- [Emergency Use \(Single Patient\) \[attach Emergency Use Checklist\]](#) (PDF)
- [Genetic Research](#) (look up "Banks, Repositories, ...Genetic/Genomic Data Sharing...")
- [Gene Transfer](#)

*For gene transfer research, also go to the E-IRB Application Other Review Committees section, and checkmark Institutional Biosafety Committee

- [International Research](#) (look up "International & Non-English Speaking")
- [NIH Genomic Data Sharing \(GDS\) Policy](#) (PDF)
- [Planned Emergency Research Involving Exception to Informed Consent*](#)

*For Planned Emergency Research Involving Waiver of Informed Consent, also go to the E-IRB Application Informed Consent section, checkmark and complete "Request for Waiver of Informed Consent Process"

- [Use, storage and disposal of radioactive material and radiation producing devices](#)

FUNDING/SUPPORT

0 unresolved
comment(s)

If the research is being submitted to, supported by, or conducted in cooperation with an external or internal agency or funding program, indicate below all the categories that apply. [i](#)

☐ Not applicable

Check All That Apply

- ☐ Grant application pending
- ☒ (HHS) Dept. of Health & Human Services
- ☒ (NIH) National Institutes of Health
- ☐ (CDC) Centers for Disease Control & Prevention
- ☐ (HRSA) Health Resources and Services Administration
- ☐ (SAMHSA) Substance Abuse and Mental Health Services Administration
- ☐ (DoJ) Department of Justice or Bureau of Prisons
- ☐ (DoE) Department of Energy
- ☐ (EPA) Environmental Protection Agency
- ☐ Federal Agencies Other Than Those Listed Here
- ☐ Industry (Other than Pharmaceutical Companies)
- ☐ Internal Grant Program w/ proposal
- ☐ Internal Grant Program w/o proposal
- ☐ National Science Foundation
- ☐ Other Institutions of Higher Education
- ☐ Pharmaceutical Company
- ☐ Private Foundation/Association
- ☐ U.S. Department of Education
- ☐ State

Click applicable listing(s) for additional requirements and information:

- [\(HHS\) Dept. of Health & Human Services](#)
- [\(NIH\) National Institutes of Health](#)
- [\(CDC\) Centers for Disease Control & Prevention](#)
- [\(HRSA\) Health Resources & Services Administration](#)
- [\(SAMHSA\) Substance Abuse & Mental Health Services Administration](#)
- Industry (Other than Pharmaceutical Companies) [[IRB Fee Info](#)-look up "Does the IRB Charge a Fee..."]
- [National Science Foundation](#)
- [\(DoEd\) U.S. Department of Education](#)
- [\(DoJ\) Department of Justice or Bureau of Prisons](#)
- [\(DoE\) Department of Energy Summary](#) and [Department of Energy Identifiable Information Compliance Checklist](#)
- [\(EPA\) Environmental Protection Agency](#)

Other:

Specify the funding source and/or cooperating organization(s) (e.g., National Cancer Institute, Ford Foundation, Eli Lilly & Company, South Western Oncology Group, Bureau of Prisons, etc.):

National Institute on Alcohol Abuse and Alcoholism

Add Related Grants

If applicable, please search for and select the OSPA Account number or Electronic Internal Approval Form (eIAF) # (notif #) associated with this IRB application using the "Add Related Grants" button.

If required by your funding agency, upload your grant using the "Grant/Contract Attachments" button.

Add Related Grants

Grant/Contract Attachments

The research involves use of Department of Defense (DoD) funding, military personnel, DoD facilities, or other DoD resources. (See [DoD SOP](#) and [DoD Summary](#) for details)

☒ Yes ☐ No

Using the "attachments" button (below), attach applicable materials addressing the specific processes described in the DoD SOP.

DOD SOP Attachments

Additional Certification: (If your project is federally funded, your funding agency may request an Assurance/ Certification/Declaration of Exemption form.) Check the following if needed:

☐ Protection of Human Subjects Assurance/Certification/Declaration of Exemption (Formerly Optional Form – 310)

Assurance/Certification Attachments

OTHER REVIEW COMMITTEES

0 unresolved
comment(s)

If you check any of the below committees, additional materials may be required with your application submission.

Does your research fall under the purview of any of the other review committees listed below? *[If yes, check all that apply and attach applicable materials using the attachment button at the bottom of your screen.]*

☐ Yes ☒ No

Additional Information

- ☐ Institutional Biosafety Committee
- ☐ Radiation Safety Committee
- ☐ Radioactive Drug Research Committee
- ☐ Markey Cancer Center (MCC) Protocol Review and Monitoring Committee (PRMC)
- ☐ Graduate Medical Education Committee (GME)
- ☐ Office of Medical Education (OME)

- [Institutional Biosafety Committee \(IBC\)](#) - Attach required IBC materials
- [Radiation Safety Committee \(RSC\)](#) - For applicability, see instructions
- [Radioactive Drug Research Committee \(RDRC\)](#)
- [Markey Cancer Center \(MCC\) Protocol Review and Monitoring Committee \(PRMC\)**](#) - Attach MCC PRMC materials, if any, per instructions.
- [Office of Medical Education \(OME\)](#)
- [Graduate Medical Education Committee \(GME\)](#)

Attachments

**** If your study involves cancer research, be sure to select "Cancer Research" in the "Research Attributes" section.** ORI will send your research protocol to the Markey Cancer Center (MCC) Protocol Review and Monitoring Committee (PRMC). The [MCC PRMC](#) is responsible for determining whether the study meets the National Cancer Institute (NCI) definition of a clinical trial and for issuing documentation to you (the investigator) which confirms either that PRMC approval has been obtained or that PRMC review is not required. Your IRB application will be processed and reviewed independently from the PRMC review.

ADDITIONAL INFORMATION/MATERIALS

0 unresolved
comment(s)Do you want specific information inserted into your approval letter? ☐ Yes ☒ No

Approval Letter Details:

If you wish to have specific language included in your approval letter (e.g., serial #, internal tracking identifier, etc...), type that language in the box below exactly as it should appear in the letter. The text you enter will automatically appear at the top of all approval letters, identical to how you typed it, until you update it. Don't include instructions or questions to ORI staff as those will appear in your approval letter. **If these details need to be changed for any reason, you are responsible for updating the content of this field.**

Additional Materials:

If you have other materials you would like to include for the IRB's consideration, check all that apply and attach the corresponding documents using the Attachments button below.

- ☐ Detailed protocol
☐ Dept. of Health & Human Services (DHHS) approved protocol (such as NIH sponsored Cooperative Group Clinical Trial)
☒ Other Documents

Protocol/Other Attachments

Attach Type	File Name
Other	Local Context Form for OSU (UKY 60446).pdf
Other	SMART IRB LOA UKY 60446 (UK-OSU).pdf
Other	Updated Communications Plan UK-OSU (60446).pdf
Other	Communications Plan UK-UM (UKY 60446)-20240251 Insomnia Intervention.pdf
Other	Local Context Form for UM (UKY 60446)-20240251 Insomnia Intervention.pdf
Other	SMART IRB LOA UKY 60446 (UK-UM).pdf
Other	hrp-216 - _form - _external_irb_reliance_application_insom interv-20240251.pdf
Other	DUA Email Confirmation.docx

NOTE: [Instructions for Dept. of Health & Human Services \(DHHS\)-approved protocol](#)]

If you have password protected documents, that feature should be disabled prior to uploading to ensure access for IRB review.

To view the materials currently attached to your application, click "All Attachments" on the left menu bar.

SIGNATURES (ASSURANCES)

0 unresolved
comment(s)

Introduction

All IRB applications require additional assurances by a Department Chairperson or equivalent (DA), and when applicable, a Faculty Advisor or equivalent (FA). This signifies the acceptance of certain responsibilities and that the science is meritorious and deserving of conduct in humans. The person assigned as DA *should not* also be listed in the Study Personnel section, and the individual assigned as FA *should* be listed in the Study Personnel section.

For a list of responsibilities reflected by signing the Assurance Statement, refer to ["What does the Department Chairperson's Assurance Statement on the IRB application mean?"](#)

For a detailed illustration of how to complete this section, please review the short online video tutorial ["Signatures \(Assurance\) Section - How to Complete."](#) Otherwise, follow the steps below.



Required Signatures:

Individuals chosen as signees may remove the application from their Inbox without signing the Assurance Statement by clicking "Return to PI" with a comment about why it is being returned (e.g., specific edits are deemed necessary).

The PI, and personnel chosen as a contact, will receive an email notification that edits are needed, and can find the draft application in both the "Draft" folder and the "Signatures Status" folder located in the menu in the left margin of the default Inbox page. The researcher does not have a 'reply' option to the signee's comments and must make the requested edits directly in the application, or communicate outside the E-IRB system as to why not. Once the response is finalized, the researcher must re-visit the "Assurances Required" section to click the "Return to Signee" button for their re-consideration; the signee will receive an email notification at that time.

Hover your mouse cursor here for additional instructions.



First Name	Last Name	Role	Department	Signee Return Comment	Date Signed	
Michelle	Martel	Department Authorization	Psychology		03/11/2024 02:15 PM	View/Sign
Justin	Verlinden	Principal Investigator	Psychology		03/01/2024 07:46 AM	View/Sign
Olga	Vsevolozhskaya	Faculty Advisor	Dept Of Biostatistics		03/06/2024 10:44 AM	View/Sign

Department Authorization

☒ This is to certify that I have reviewed this research protocol and that I attest to the scientific validity and importance of this study; to the qualifications of the investigator(s) to conduct the project and their time available for the project; that facilities, equipment, and personnel are adequate to conduct the research; and that continued guidance will be provided as appropriate. When the principal investigator assumes a sponsor function, the investigator has been notified of the additional regulatory requirements of the sponsor and by signing the principal investigator Assurance Statement, confirms he/she can comply with them.

*If the Principal Investigator is also the Chairperson of the department, the Vice Chairperson or equivalent should complete the "Department Authorization".

**IF APPLICABLE FOR RELIANCE: I attest that the principal investigator has been notified of the regulatory requirements of both the Reviewing and Relying IRBs, according to the information provided in the E-IRB application. The attached Reliance Assurance Statement, signed by the principal investigator, confirms that he/she can comply with both sets of IRB requirements.

Principal Investigator's Assurance Statement

I understand the University of Kentucky's policies concerning research involving human subjects and I agree:

1. To comply with all IRB policies, decisions, conditions, and requirements;
2. To accept responsibility for the scientific and ethical conduct of this research study;
3. To obtain prior approval from the Institutional Review Board before amending or altering the research protocol or implementing changes in the approved consent/assent form;
4. To report to the IRB in accord with IRB/IBC policy, any adverse event(s) and/or unanticipated problem(s) involving risks to subjects;
5. To complete, on request by the IRB for Full and Expedited studies, the Continuation/Final Review Forms;
6. To notify the Office of Sponsored Projects Administration (OSPA) and/or the IRB (when applicable) of the development of any financial interest not already disclosed;
7. Each individual listed as study personnel in this application has received the mandatory human research protections education (e.g., CITI);
8. Each individual listed as study personnel in this application possesses the necessary experience for conducting research activities in the role described for this research study.
9. To recognize and accept additional regulatory responsibilities if serving as both a sponsor and investigator for FDA regulated research.

☒ Furthermore, by checking this box, I also attest that:

- I have appropriate facilities and resources for conducting the study;
- I am aware of and take full responsibility for the accuracy of all materials submitted to the IRB for review;
- If applying for an exemption, I also certify that the only involvement of human subjects in this research study will be in the categories specified in the Protocol Type: Exemption Categories section.
- If applying for an Abbreviated Application (AA) to rely on an external IRB, I understand that certain items above (1, 3, 4, 7-8) may not apply, or may be altered due to external institutional/IRB policies. I document my agreement with the [Principal Investigator Reliance Assurance Statement](#) by digitally signing this application.

***You will be able to "sign" your assurance after you have sent your application for signatures (use Submission section). Once all Assurance Statement signatures have been acquired, return to this section to submit your application to ORI.**

Faculty Advisor's Assurance Statement

☒ This is to certify that I have reviewed this research protocol and that I attest to the scientific merit of this study; to the qualifications of the investigator(s) to conduct the project; that facilities, equipment, and personnel are adequate to conduct the research; and that continued guidance will be provided as appropriate.

****If the Principal Investigator is completing this project to meet the requirements of a University of Kentucky academic program, in addition to Department Authorization, the student's faculty advisor should sign the Assurance Statement. The student's faculty advisor is accepting a supervisory role in guiding the student in conducting regulatory compliant research and therefore must be certified in human research protection training throughout the life of the protocol.**

SUBMISSION INFORMATION

0 unresolved
comment(s)

*** If this Continuation Review entails a change in the scope of your activities to include COVID-19 related research, please insert "COVID19" at the start of your Project and Short Titles.***

Each Section/Subsection in the menu on the left must have a checkmark beside it (except this Submission section) indicating the Section/Subsection has been completed. Otherwise your submission for IRB review and approval cannot be sent to the Office of Research Integrity/IRB.

If applicable, remember to update the Approval Letter Details text box under the Additional Information section

If your materials require review at a convened IRB meeting which you will be asked to attend, it will be scheduled on the next available agenda and you will receive a message to notify you of the date.

If you are making a change to an attachment, you need to delete the attachment, upload a highlighted version that contains the changes (use Document Type of "Highlighted Changes"), and a version that contains the changes without any highlights (use the appropriate Document Type for the item(s)). Do **not** delete approved attachments that are still in use.

Principal Investigator's Assurance Statement

I understand the University of Kentucky's policies concerning research involving human subjects, and I attest to:

1. Having reviewed all the investigational data from this study, including a compilation of all internal and external unanticipated problems.
2. Having reviewed, if applicable, information from the sponsor including updated investigator brochures and data and safety monitoring board reports.

I also attest that I have reviewed pertinent materials concerning the research and concluded:

- The human subject risk/benefit relationship is unaffected, mitigated, or eliminated by closure of the study and all pertinent materials for closure of the research are being submitted to the IRB for consideration.

☒ By checking this box, I am providing assurances for the applicable items listed above.

Your protocol has been submitted.

Download all

	Document Type	File Loaded	Document Description	File Size	Modified By	Mod Date
	StudyClosure	StudyClosure.pdf		0.079	mcgies2	7/16/2025 5:04:56 PM
	AddInfoProduct	DUA Email Confirmation.docx	Screenshot of email confirmation of DUA	0.266	jjve226	8/28/2024 11:51:58 AM
	AddInfoProduct	hrp-216_-_form_-_external_irb_reliance_application_insom interv-20240251.pdf	UM External Reliance Application	0.360	jjve226	5/30/2024 10:18:18 AM
	AddInfoProduct	SMART IRB LOA UKY 60446 (UK-UM).pdf	SMART IRB LOA UK (UK-UM)	0.240	jjve226	5/30/2024 10:17:35 AM
	AddInfoProduct	Local Context Form for UM (UKY 60446)-20240251 Insomnia Intervention.pdf	Reliance Agreement - Local Context Form (UK-UM)	0.281	jjve226	5/30/2024 10:16:59 AM
	AddInfoProduct	Communications Plan UK-UM (UKY 60446)-20240251 Insomnia Intervention.pdf	Communications Plan UK-UM	0.420	jjve226	5/30/2024 10:16:33 AM
	AddInfoProduct	Updated Communications Plan UK-OSU (60446).pdf	Updated Communications Plan	0.306	ecgu227	3/26/2024 9:22:16 AM
	DataCollection	SIESTA Post-intervention Interview Protocol 3-11-23.docx	Up-to-Date End of Study Phone Interview Protocol	0.030	jjve226	3/11/2024 1:39:52 PM
	Advertising	Ohio-BCFS00690-UK-Moloney-AUD-AdCopy (1)-STAMPED.pdf	Recruitment Language for OH	0.111	ecgu227	11/13/2023 12:04:05 PM
	Advertising	Kentucky-BCFS00690-UK-Moloney-AUD-AdCopy (1)-STAMPED.pdf	Recruitment Language for KY	0.111	ecgu227	11/13/2023 12:03:54 PM
	Advertising	Indiana-BCFS00690-UK-Moloney-AUD-AdCopy (1)-STAMPED.pdf	Recruitment Language for IN	0.111	ecgu227	11/13/2023 12:03:44 PM
	Advertising	BCFS00690-UK-Moloney-AUD-Feed-STAMPED.pdf	Feed	10.463	ecgu227	11/13/2023 12:03:26 PM
	Advertising	BCFS00690-UK-Moloney-AUD-Ad2-STAMPED.pdf	Ads	0.102	ecgu227	11/13/2023 12:03:18 PM
	Advertising	BCFS00690-UK-Moloney-AUD Screening Form-STAMPED.pdf	Screening Form	0.176	ecgu227	11/13/2023 12:03:07 PM
	Advertising	BCFS00690-UK-Moloney-AUD [Landing Page]-STAMPED.pdf	Landing Page	0.674	ecgu227	11/13/2023 12:02:58 PM
	AddInfoProduct	SMART IRB LOA UKY 60446 (UK-OSU).pdf	Reliance Agreement - SMART IRB LOA	0.189	ecgu227	9/7/2023 10:22:10 AM
	AddInfoProduct	Local Context Form for OSU (UKY 60446).pdf	Reliance Agreement - Local Context Form	0.235	ecgu227	9/7/2023 10:21:29 AM
	DataCollection	SIESTA prescreen final.pdf		0.043	kcwe225	6/1/2023 10:20:56 AM
	Advertising	PSYCH-071b_MON-STAMPED.pdf		0.164	kcwe225	3/28/2023 11:11:55 AM
	Advertising	PSYCH-071b_flyer-STAMPED.pdf		0.414	kcwe225	3/28/2023 11:11:46 AM
						3/28/2023

					60446 CLOSED	
⚡	Advertising	PSYCH-071b social media-STAMPED.pdf		0.156		AM
⚡	Advertising	PSYCH-071b researchmatch-STAMPED.pdf		0.095	kcwe225	3/28/2023 11:11:31 AM
⚡	DataCollection	STOP-Bang.docx	STOP-Bang & GUID Info (Collected during TLFB)	0.014	jjve226	2/16/2023 2:40:04 PM
⚡	DataCollection	TLFB Zoom Instructions 2.16.23.docx	Instructional Sheet for TLFB	0.028	jjve226	2/16/2023 2:39:29 PM
⚡	Advertising	PSYCH-071a_MON-STAMPED.pdf		0.092	kcwe225	1/6/2023 12:31:12 PM
⚡	Advertising	PSYCH-071a_flyer-STAMPED.pdf		0.399	kcwe225	1/6/2023 12:31:04 PM
⚡	Advertising	PSYCH-071a social media-STAMPED.pdf		0.155	kcwe225	1/6/2023 12:30:49 PM
⚡	Advertising	PSYCH-071a researchmatch-STAMPED.pdf		0.093	kcwe225	1/6/2023 12:30:42 PM
⚡	DataCollection	Sleep_Med_Questionnaire.pdf		0.017	jjve226	10/11/2022 10:36:13 AM
⚡	DataCollection	9. APT-Trait V1 20 NOV 2019.docx		0.016	jjve226	10/11/2022 10:30:28 AM
⚡	DataCollection	8.Composite Morningness Questionnaire.pdf		0.309	jjve226	10/11/2022 10:30:10 AM
⚡	DataCollection	7.Treatment_Satisfaction.docx		0.018	jjve226	10/11/2022 10:29:53 AM
⚡	AdditionInfoConsiderations	Layne Robinson Citi Certificate sept 2021.pdf	citi completion report for Layne	0.401	jblac2	9/8/2021 3:48:25 PM
⚡	AdditionInfoConsiderations	Caitlyn completed University of Kentucky Assumption of Risk and Waiver of Liability.pdf	assumption of risk for caitlyn	0.238	jblac2	9/3/2021 12:18:33 PM
⚡	AdditionInfoConsiderations	Caitlyn completed Emergency contact form.pdf	emergency contact for Caitlyn	0.045	jblac2	9/3/2021 12:18:18 PM
⚡	AdditionInfoConsiderations	Caitlyn completed Informed Consent Voluntary Waiver Release of Liability and Assumption of Risk Form_0.pdf	informed consent for Caitlyn	0.468	jblac2	9/3/2021 12:18:02 PM
⚡	AdditionInfoConsiderations	citiCompletionReport10315630 (3).pdf	citi completion report for Caitlyn	0.400	jblac2	9/3/2021 12:17:44 PM
⚡	AdditionInfoConsiderations	IIA for Caitlyn Smith 60446.pdf	IIA for Caitlyn Smith	0.170	jblac2	9/3/2021 12:17:03 PM
⚡	AdditionInfoConsiderations	UKCampWaiver-localr (1) for minors.pdf		0.371	rcvaug2	8/10/2021 11:57:15 AM
⚡	AdditionInfoConsiderations	what-constitutes-study-personnel-protocol- involving-human-subjects-pdf.pdf		0.019	rcvaug2	8/10/2021 11:57:05 AM
⚡	Advertising	PODS PSYCH-071 research match PR STAMPED irb 60446.pdf	research match advertisement	0.048	jblac2	7/22/2021 11:54:31 AM
⚡	Advertising	PODS flyer QR code PR edit STAMPED.pdf	updated stamped flyer	0.347	jblac2	2/8/2021 10:34:10 AM

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	Advertising	PODS flyer_v2_PR_approved.pdf		0.393	jweaf2	7/21/2020 10:33:23 AM
	DataCollection	CESD-10.pdf		0.248	jweaf2	6/25/2020 7:50:39 PM
	DataCollection	PerceivedStressScale.pdf		0.234	jweaf2	6/25/2020 7:50:30 PM
	DataCollection	TLFB.docx		0.045	jweaf2	6/25/2020 7:50:13 PM
	DataCollection	AUDIT.docx		0.071	jweaf2	6/25/2020 7:50:05 PM
	DataCollection	PSQI_sleep_questionnaire_1_pg.pdf		0.110	jweaf2	6/25/2020 7:49:48 PM
	DataCollection	Insomnia Severity Index -ISI.pdf		0.030	jweaf2	6/25/2020 7:49:36 PM

Protocol Changes

Click link to sort [Changed Date](#)
HIPAA HIPAAIdentificationCertForm changed by jive226 on 7/7/2025 1:29:50 PM
N
Informed Consent ElectronicConsent changed by jive226 on 7/7/2025 1:29:31 PM
Y
Research Attributes EpidemiologicBehavioralStudies changed by jive226 on 7/7/2025 1:31:01 PM
Y
Research Attributes MaterialOfHumanOrigin changed by jive226 on 7/7/2025 1:31:01 PM
N
Research Attributes OutcomesHealthServicesResearch changed by jive226 on 7/7/2025 1:31:01 PM
Y
Research Attributes PatientOrientedResearch changed by jive226 on 7/7/2025 1:31:01 PM
Y
Research Sites MultisiteLeadInvestigator changed by jive226 on 7/7/2025 1:30:00 PM
*

Study Personnel Changes:

Status	PIIdentity	ProtocolID	PersonID	RoleInProtocol	IsContact	LastName	FirstName	Email	DeptCode	RoomBuilding	SpeedSort	PhoneNum	DeptDesc	AuthorizedConsent	ResponsibilityInProject	Degree	Rank	StatusFlag	IsRemoved	ModBy	ModDate	SFI	IsPIRN	MiddleName
Updated	1066884	106178	12121317	PI	Y	Verlinden	Justin	justinverlinden@uky.edu	8E120	Kastle Hall 2070	0044	3099484850	Psychology		Principal Investigator	MS			N	jive226	7/7/2025 1:26:59 PM	N	N	James

Protocol Type Comment by Mary Gieske - ORI to PI on 7/8/2025 4:39:23 PM

Select "All Comments" from the left-hand sidebar to see all the pre-review screening comments from ORI staff.

Please be aware that the IRB has not yet seen your application. This is an initial screening to ensure your application is ready for IRB review. After you review the screening requests and your application appears to be ready, it will be forwarded to the IRB for review. The IRB may request additional information or clarifications after their review.

If you have any questions regarding these comments, please contact me by email or phone (katie.gieske@uky.edu, 859-257-3138).

Continuation/Final Review Comment by Justin Verlinden - PI to PI on 7/9/2025 1:00:22 PM

To clarify, all data used for analysis does not have identifiable information contained. However, we still have access to encrypted identifiers (it is just separate from the actual data). With this in mind, did I choose the correct button or should I choose the button below, "There is no subject/record/specimen identifying codes or links to the data". Either way, I can confirm that all identifiable information is encrypted.

Continuation/Final Review Comment by Mary Gieske - ORI to PI on 7/8/2025 4:37:26 PM

The PI has selected the remaining activities are limited only to data analysis and there IS access to records either directly or through codes or links to the data. Is your data (with identifiers) encrypted (i.e., converted into a code to prevent unauthorized access)?

The study may be closed if the data with identifiers, is encrypted.